Strengthening legal and regulatory frameworks for maternal and perinatal death surveillance and response

World Health Organization

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Abbreviations

CEDAW	Convention on the Elimination of All Forms of Discrimination Against Women
CEDAW Committee	Committee on the Elimination of Discrimination against Women
CESCR	Committee on Economic, Social, and Cultural Rights
CRC	Convention on the Rights of the Child
CRC Committee	Committee on the Rights of the Child
CRVS	Civil Registration and Vital Statistics
FIMRC	Fetal and Infant Mortality Review Committee
ICD	International Classification of Diseases
ICCPR	International Covenant on Civil and Political Rights
ICESCR	International Covenant on Economic, Social and Cultural Rights
MMRC	Maternal Mortality Review Committee
MPDSR	Maternal and perinatal death surveillance and response
NGO	Nongovernmental organization
UN DESA	United Nations Department of Economic and Social Affairs
WHO	World Health Organization

Executive summary

This manual for strengthening legal and regulatory frameworks for maternal and perinatal death surveillance and response (MPDSR) aims to support various state actors in enhancing legal and regulatory frameworks for MPDSR. MPDSR is "an essential quality improvement intervention which permits the identification, notification, quantification and determination of causes and avoidability of maternal and neonatal deaths and stillbirth with the goal of orienting the measures necessary for their prevention". *(1)* Strengthening frameworks may involve applying existing laws and regulations to MPDSR, developing new laws or regulations, or amending current ones to establish a legal and regulatory mandate for the creation and implementation of MPDSR. This manual emphasizes the importance of a structured approach to monitoring and responding to maternal and perinatal deaths, aiming to enhance accountability—including legal accountability—to enable quality improvements and reduce preventable deaths. Establishing MPDSR frameworks not only ensures a coordinated response to these deaths but also highlights the intrinsic link between maternal and perinatal health, setting the stage for integrated strategies that address both maternal and perinatal deaths comprehensively.

Integrated approach: linking maternal and perinatal deaths

Maternal and perinatal deaths are intrinsically linked, with many perinatal deaths resulting from maternal conditions and complications. Addressing these deaths in isolation overlooks critical connections and underlying causes, often led by socio-cultural factors. Stillbirths fall within the definition of perinatal deaths; however, stillbirths often go uncounted in many countries. This manual emphasizes the need for strong advocacy and legal accountability to include stillbirths in MPDSR laws and regulations. A holistic MPDSR framework allows for comprehensive analysis and intervention strategies that target both maternal and perinatal health.

The necessity of a legal and regulatory framework for MPDSR

Maternal and perinatal deaths remain significant public health challenges worldwide, with considerable regional disparities and sharp contrasts at the national level depending on the socio-economic background. A robust framework for MPDSR is essential to ensure systematic and consistent surveillance, reporting, and response to these deaths. Laws and regulations provide the necessary authority and mandate for healthcare systems and communities to collect accurate data, analyse causes, and implement interventions. It also ensures that all stakeholders are accountable and committed to improving maternal and perinatal health outcomes.

Integrating the manual into the legal or regulatory process

Integrating this manual into the legal or regulatory process means establishing a clear, standardized framework for MPDSR that is backed by legal authority. This involves the following components:

- **Development of laws and regulations**: Drafting and enacting laws or regulations that mandate the collection, reporting, and analysis of maternal and perinatal deaths, including stillbirths, while taking into account the underlying causes for these deaths and addressing them in the legal and regulatory frameworks.
- **Capacity building**: Training healthcare providers, policymakers, and other stakeholders on the requirements and processes of MPDSR.
- **System strengthening**: Enhancing health information systems to support accurate data collection and analysis.

- **Stakeholder engagement**: Fostering collaboration among government agencies, healthcare providers, and communities to ensure widespread support and compliance, while developing sustained strategies to positively transform mindsets.
- **Accountability mechanisms**: Establishing clear lines of accountability to ensure that data collected leads to actionable insights and interventions aimed at reducing maternal and perinatal deaths.
- **Remedies, redress, and disciplinary and liability mechanisms**: MPDSR laws and regulations can advise on the existence of separate remedial and redress mechanisms for families and communities, as well as independent disciplinary and liability mechanisms for healthcare workers.

This manual may serve as an essential tool to guide the development of legal or regulatory MPDSR frameworks, enabling countries to establish a robust and accountable system for systematically reviewing and addressing all maternal and perinatal deaths. By fostering a comprehensive approach, it aims to enable quality improvements, reduce preventable deaths, and strengthen interventions to improve maternal and newborn health outcomes.

Introduction

Although recent global and national efforts have yielded significant progress, reducing maternal and perinatal mortality remains a challenge, in particular in the most precarious socio-economic contexts. Establishing an accountable, legally mandated national maternal and perinatal death surveillance and response (MPDSR) system, consistent with the state's constitutional provisions, human rights obligations, and public health commitments, can be a significant step towards improving quality of care and ending preventable maternal and perinatal mortality.

Laws and regulations are essential for strengthening the accountability of MPDSR systems, a key quality improvement intervention. Accountability can be achieved by leveraging existing laws or regulations, amending current ones, or by developing new legal or regulatory frameworks, with the most effective approach tailored the specific national context. The guidance contained in the manual is adaptable to various national contexts and focusses on building the capacity of all actors involved in the MPDSR legal or regulatory process. The manual supports a legal or regulatory framework for MPDSR, in alignment with health standards and human rights obligations, standards, and guidance, which may inform the development and content of laws or regulations.

Historically, countries have primarily relied on policy to implement MPDSR. Despite the growing adoption of national policies to review maternal and perinatal deaths, a significant gap in effective policy implementation remains. Countries may find that laws and regulations serve as a valuable accountability tool to supplement policy, particularly by establishing a legal or regulatory mandate for reporting and reviewing maternal and perinatal deaths. Apart from promoting accountability and transparency by revealing the magnitude of maternal and perinatal deaths currently underreported in vital registration systems, the normative force of law establishes a clear chain of enforceable institutional responsibilities for reporting. This enhances the 'surveillance' and 'response' components of MPDSR, which in turn enables improvements in the quality of care. Laws and regulations also facilitate the creation of an institutional framework with legal powers and duties to conduct surveillance, collate information, review deaths and recommend timely interventions addressing modifiable factors in facilities and communities.

Fear of blame is a significant deterrent, preventing many healthcare workers and related personnel from participating in MPDSR processes. Laws and regulations can provide legal protection for individuals who participate in maternal and perinatal death reviews by ensuring the confidentiality of data and by protecting participants from any legal claim related to information from the review process. Additionally, laws and regulations can strengthen policy by making the reporting of maternal and perinatal deaths mandatory rather than discretionary, and by establishing institutional structures and frameworks accountable for reviewing those deaths. They can also secure sustained funding for the MPDSR process, especially important in low-resource settings where other programmes compete for financial support.

It is important to stress that laws and regulations alone are not sufficient to ensure the efficacy of a legal or regulatory framework for MPDSR. Efficacy depends on a variety of factors, including local capacity to develop, implement and enforce legal or regulatory mechanisms, budgetary allocations to ensure that the MPDSR system is adequately resourced and, ultimately, the political will and commitment of the state to act on the recommendations of MPDSR review committees.

This manual comprises three sections. Part 1, Scope and Objectives, provides an overview of the key concepts and objectives of MPDSR laws and regulations, along with an explanation of its importance. Part 2, the Checklist, offers guidance on the questions to consider and the steps to follow when reviewing, developing and/or strengthening MPDSR laws and regulations. Part 3, the Process Guide, highlights the important steps to follow when undertaking such a legal or regulatory process.

The manual is guided by existing public health standards and human rights obligations, standards, and guidance for the protection of mothers and newborns. Particularly, it seeks to complement technical guidance on establishing and carrying out MPDSR processes developed by the World Health Organization (WHO) in the last decade, including: *Maternal death surveillance and response: technical guidance information for action to prevent maternal death* (2013); *(2) Every Newborn: an action plan to end preventable deaths* (2014); *(3) Making every baby count: audit and review of stillbirths and neonatal deaths* (2016); *(4) Maternal and perinatal death surveillance and response: materials to support implementation* (2021); *(5)* and *the International Classification of Diseases (ICD) 11 reference guide* (2022). *(6)*

This manual reflects human rights obligations contained in human rights treaties and the clarification of substantive human rights obligations developed by international and regional human rights treaty bodies, institutions, and legal or regulatory bodies. This includes General Comments from the Committee on Economic Social and Cultural Rights (CESCR), General Comments from the Committee on the Rights of the Child (CRC Committee), and General Recommendations from the Committee on the Elimination of Discrimination against Women (CEDAW Committee), the African Union Commission policy documents and jurisprudence from the Inter-American Court of Human Rights. National interpretations of human rights obligations are also addressed. Additionally, the manual considers various technical guidance documents related to maternal and perinatal mortality. *(7,8,9,10)*

Key outcomes of MPDSR laws and regulations

MPDSR laws and regulations aim to enable quality of care improvements and reduce maternal and perinatal mortality by creating a systematic approach to review and respond to each death. The key outcomes of implementing MPDSR laws and regulations include the following:

1. **Improved data collection and surveillance:**
   - Systematic reporting: Establishes mandatory reporting systems for maternal and perinatal deaths, ensuring comprehensive data collection.
   - Standardized definitions: Ensures uniform definitions and criteria for maternal and perinatal deaths, enhancing data accuracy and comparability.
2. **Enhanced accountability:**
   - Regular reviews: Mandates regular reviews and audits of maternal and perinatal deaths to identify patterns and causes.
   - Clear responsibilities: Clarifies roles and responsibilities of healthcare providers, facilities, and authorities in monitoring and responding to these deaths.
3. **Informed legal and regulatory development:**
   - Evidence-based interventions: Provides data-driven insights to inform laws, regulations, policies, and resource allocation aimed at reducing mortality rates, including data documenting the socio-cultural barriers to women's and children's access to the highest attainable standard of physical and mental health.
   - Targeted actions: Identifies gaps in healthcare delivery and areas needing improvement, guiding targeted interventions.
4. **Quality improvement in healthcare services:**
   - Best practices: Promotes the adoption of best practices and protocols in maternal and newborn care.

- Training and capacity building: Enhances the training and capacity of healthcare providers through feedback and continuous education.

5. Community and stakeholder engagement:

- Awareness and education: Raises awareness among communities and stakeholders about maternal and newborn health issues.
- Collaboration: Encourages collaboration among government agencies, healthcare providers, non-governmental organizations, and communities.

6. Reduction in maternal and perinatal mortality:

- Preventive measures: Identifies preventable factors and implements measures to avoid future deaths.
- Timely interventions: Facilitates timely and appropriate responses to complications during pregnancy, childbirth, and the postpartum period.

7. Legal and ethical framework:

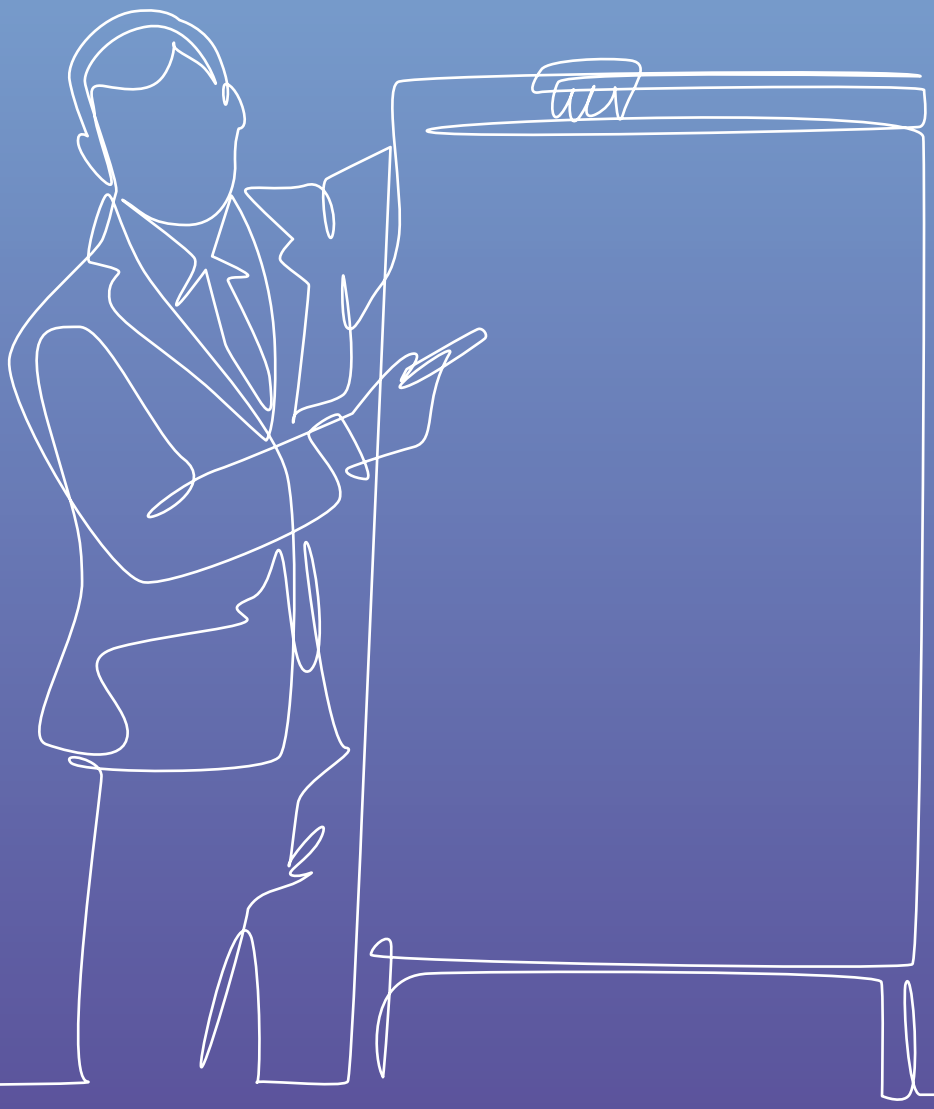
- Confidentiality: Ensures confidentiality and ethical handling of information and data collected in the MPDSR process.
- No-blame approach: Promotes a no-blame approach to reporting and reviewing deaths, encouraging honest and open participation from healthcare providers.
- MPDSR systems that advise on separate remedial and redress mechanisms for families and communities, as well as independent disciplinary and liability mechanisms for healthcare workers.

8. Sustainable health system strengthening:

- Long-term improvements: Contributes to long-term improvements in the health system, making it more resilient and responsive to maternal and newborn health needs.
- International obligations, standards and guidance: Aligns with international obligations, standards, and guidance including WHO MPDSR guidance and human rights.

These outcomes collectively aim to create a comprehensive and responsive system that enables quality improvements and reduces maternal and perinatal mortality through continuous monitoring, analysis, and targeted actions. Implementation of MPDSR systems should be guided by the WHO's *Maternal and perinatal death surveillance and response: materials to support implementation* (2021) and national MPDSR guidelines or policies. *(11)*

This manual for strengthening legal and regulatory frameworks for MPDSR is intended to provide general principles that may be applicable, depending on context, to facilitate the establishment of a national framework on MPDSR. With this in mind, this manual is not drafted or intended to address any specific national legislative arrangement, is provided for reference only, and does not constitute legal advice or a legal opinion on the part of WHO on any specific domestic laws or jurisdiction. This manual should therefore not be referred to, directly or indirectly, as constituting WHO's legal advice or legal opinion before any court, tribunal, or proceeding. Furthermore, while WHO has taken reasonable steps to verify the accuracy and completeness of information provided in this manual, WHO is not in a position to guarantee that this manual is complete or accurate, or appropriate for any specific use or matter. Finally, throughout this manual, references are made to human rights standards in international law, the applicability of which in a specific setting will depend on factors such as the State's status as a signatory or its ratification of relevant human rights instruments.



Part 1: Scope and objectives

1. Introduction

Safe motherhood is a human rights imperative involving specific freedoms and entitlements, yet, preventable maternal mortality persists. *(12,13,14,15)* An estimated 287,000 maternal deaths occur globally each year. *(16)* In addition, 2.3 million perinatal deaths and 1.9 million stillbirths occur annually. *(17)* The overwhelming majority (95%) of maternal and neonatal deaths occur in low-resource settings and among poor and marginalized women. *(18)* High global perinatal mortality remains a major public health and human rights concern. *(19,20,21)* The greatest burden of perinatal mortality (95%) falls on low- and middle-income countries, with the largest numbers of stillbirths and neonatal deaths occurring in southern Asia and sub-Saharan Africa. *(22)* A public health approach to reducing maternal and perinatal deaths grounded in human rights requires the state, state actors and other relevant stakeholders to take action at all levels to address the interlinked central causes of preventable mortality.

In the context of the Sustainable Development Goals, countries have united behind targets to accelerate the decline of maternal and perinatal mortality. The goals include reducing global maternal mortality to less than 70 per 100,000 live births and ending preventable deaths of newborns by reducing neonatal mortality to at least as low as 12 per 1,000 live births by 2030. *(23)* These objectives align with the targets set forth in the WHO's 2014 *Every Newborn: an* action plan *to end preventable deaths*. *(24)*

Countries recognize, through international, regional, and national laws, regulations, and policies, that improving the health of women and newborns, as well as reducing maternal and perinatal mortality and morbidity, requires the development, implementation, and monitoring of a comprehensive system. This system should include enabling laws, regulations, policies, programmes, and health systems within a social and political environment that protects the rights of women and newborns and fosters accountability. *(25,26,27,28,29,30,31)* MPDSR is a component of such a system. MPDSR improves maternal and perinatal health and fosters accountability by reducing preventable maternal and perinatal mortality through a continuous cycle of notification, review, analysis, and response. *(32)*

WHO defines MPDSR as "an essential quality improvement intervention which permits the identification, notification, quantification and determination of causes and avoidability of maternal and neonatal deaths and stillbirth with the goal of orienting the measures necessary for their prevention". *(33)*

The efficacy of MPDSR relies on the following processes:

- effective identification and reporting of maternal and perinatal deaths;
- determination of the causes of these deaths with the aim of identifying preventative measures;
- assignment of these preventative measures to specific groups or individuals with designated time frames for completing these actions as part of quality improvement interventions; and
- follow-up to ensure that preventative actions have been implemented and corrective measures have been secured.

MPDSR is a quality improvement intervention designed to reduce preventable maternal and perinatal mortality by engaging relevant stakeholders throughout the process. This involves identifying maternal and perinatal deaths that occur during pregnancy, around the time of childbirth or in the postnatal period.

By understanding and documenting the reasons behind these deaths, MPDSR facilitates steps to prevent future occurrences. Crucial to the effectiveness of MPDSR is the protection of confidentiality and establishment of a blame-free culture. MPDSR systems must address the fear of blame, disciplinary proceedings, and lawsuits that may deter healthcare workers from participating. The primary objective of MPDSR is not to assign individual blame but enhance maternal and perinatal healthcare. This is achieved by collecting and interpreting aggregated data, providing insights, and recommending preventative actions within an environment that fosters accountability.

The manual addresses accountability by ensuring that MPDSR systems are responsive to relevant stakeholders, including parliamentarians, government sectors, professional associations and civil society. It focusses on establishing enabling legal frameworks, thereby connecting legal accountability with other forms of accountability, such as those related to the health system and social accountability. For example, it outlines the necessity of laws and regulations that incorporate health standards and human rights obligations, standards, and guidance and emphasizes that these instruments should consider health system needs. The manual advocates for the involvement of civil society and other non-state actors in legal and regulatory processes and the implementation of MPDSR. This may include women's and children's groups, Indigenous Peoples, women belonging to ethnic or racial minority groups and women with disabilities.

The manual provides a detailed explanation of how laws and regulations can facilitate a no-blame approach by ensuring confidentiality and data protection. It also outlines legal protections for health care workers, enabling them to carry out surveillance and response without fear of blame or prosecution based on the data they provide within the MPDSR context. Additionally, the manual emphasizes that MPDSR systems should specifically focus on providing assurances regarding access to remedies and redress for affected individuals and healthcare workers through procedures external to MPDSR. It highlights the necessity for remedy and redress mechanisms, as well as disciplinary and liability proceedings, to co-exist alongside, yet remain separate from MPDSR systems.

The manual acknowledges that the overwhelming majority of countries have thus far relied on policy, rather than laws or regulations, to establish MPDSR systems. However, some countries have an established history of using law as a component of MPDSR, while others are adopting law to supplement policy in implementing MPDSR. Other countries have developed maternal death-related jurisprudence through litigation and judicial decisions. A few countries have enacted laws to establish committees obligated to conduct maternal and perinatal death reviews. The manual also notes that at the implementation level, the surveillance component of MPDSR is more developed for maternal death surveillance and response than it is for perinatal death surveillance and response, indicating that further development is needed in this area.

Figure 1 below provides an overview of the manual's contents. It illustrates the key components, processes, and necessary steps for developing a MPDSR legal and regulatory framework. The manual is divided into three parts:

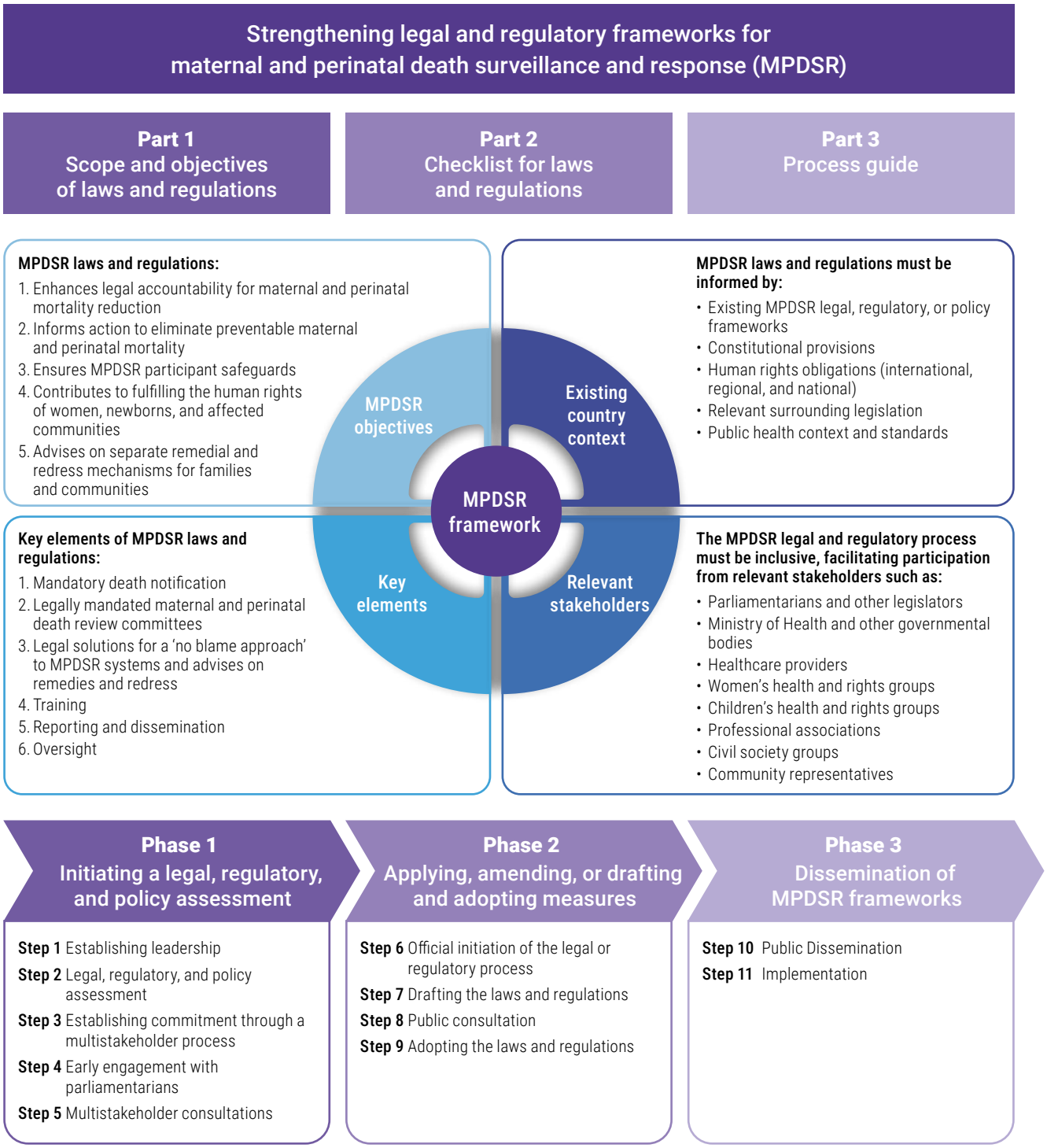
Part 1 outlines the scope and objectives of MPDSR laws and regulations.

Part 2 provides a checklist for assessing the quality of current or new laws, regulations, or policies.

Part 3 offers a step-by-step guide to the legal and regulatory process.

Figure 1 highlights the key components within each part, including the objectives of MPDSR laws and regulations, the relevance of current legal and policy frameworks, key stakeholders, and essential elements for effective MPDSR laws and regulations. Additionally, it reflects the three legal and regulatory phases detailed in the manual. The legal and regulatory process is divided into eleven key steps, each associated with a specific phase.

Figure 1: Strengthening legal and regulatory frameworks for MPDSR



2. Methodology and definitions

The development of the manual was based on a desk review and pilot testing of the draft manual in various countries that have already implemented, or aim to implement, a legal or regulatory framework for an MPDSR system. The aim was not to study all countries with an MPDSR system but to capture a representative sample of those that have introduced laws or regulations as a means of implementing their MPDSR systems, replacing the historical reliance on policy as the primary MPDSR tool. The main criteria for selecting countries for the study were geographical diversity, a history of using laws or regulation to implement MPDSR, a history of maternal and perinatal death-related litigation, the development of maternal and perinatal death-related jurisprudence, domestic proposals to introduce laws and regulations for MPDSR systems, and interest in using laws and regulations to implement MPDSR.

This manual was subject to an initial desk review of relevant literature on existing MPSDSR frameworks in various countries and legal and policy documents obtained from WHO Member States through the WHO Global Sexual, Reproductive, Maternal, Newborn, Child and Adolescent Health (SRMNCAH) Policy Survey. This manual was also subject to pilot tests in a select number of countries where the reduction of maternal and perinatal mortality and morbidity is a recognized public health concern to test its viability and assist countries in strengthening the legal accountability of their MPDSR systems. An additional objective was to gain further insight into legally ensuring a no-blame approach for the MPDSR process. The pilot tests were based on literature reviews and critical legal analyses to assess existing MPDSR frameworks. The relevant constitutional, human rights and general legal frameworks were also assessed. The assessment and analysis relied on the process and guidance contained in this manual. The desk review and the pilot tests assessed MPDSR frameworks in high-income, middle-income, and low-income countries. The manual was refined based on feedback from pilot test countries.

The manual provides an overview of several types of current global developments of domestic MPDSR systems, including MPDSR laws, regulations and policies. However, it is not exhaustive. Examples are primarily descriptive rather than evaluative, as they do not seek to comprehensively assess the efficacy of laws, regulations or policies.

2.1 Definitions

2.1.1 Maternal death period and perinatal death period

Definitions of maternal death and perinatal death have implications for consistent reporting and the collation of data. For the purposes of the manual and MPDSR, the concepts of maternal death and perinatal death follow the guidance of WHO and the ICD-11. *(34)*

Maternal death period

Maternal death means the death of a woman while pregnant or within 42 days of the termination of pregnancy, irrespective of the duration and site of the pregnancy, from any cause related to or aggravated by the pregnancy or its management, but not from accidental or incidental causes. *(35)*

Perinatal death period

Perinatal death review and surveillance covers both stillbirths and neonatal deaths. *(36,37,38,39)* MPDSR requires the use of universalized criteria for reporting stillbirths and the perinatal period covered. According to the WHO ICD-11, the perinatal period lasts from 22 completed weeks of gestation to seven completed days of postnatal life. *(40)*

Stillbirths are categorized as occurring either before the onset of labour (antepartum) or after the onset of labour but prior to childbirth (intrapartum). The term '**stillbirth**' refers to the delivery of a dead fetus at 22 weeks' gestation or later while a **neonatal death** is a death after live birth which occurs during the neonatal period. The WHO ICD 11 Reference Guidance refers to birth weight, body length and gestational age in its definition of a reportable stillbirth. At a minimum, all stillbirths and deaths following live births born with 22 or more completed weeks of gestation (≥ 154 days) should be reported to the MPDSR system and included in the statistics, though the legal requirements for the registration may vary depending on different national legal or regulatory frameworks. A stillbirth reported in the MPDSR system and in the statistics must involve one of the following:

- a fetus with a birth weight of at least 500 g; or
- if birth weight is unavailable, a fetus that has reached either the gestational age of at least 22 weeks or 25 cm in body length from crown-to-heel.

A fetal death with a birth weight of 500–1000 g or at 22–28 weeks' gestation is classified as an early fetal death. A fetal death with a birth weight greater than 1000 g or over 28 weeks' gestation is a late fetal death.

For gathering statistics at the national level, WHO recommends that all deaths involving fetuses or infants weighing 500 g or more should be reported. Because birth weight information might be unavailable for stillbirths, WHO recommends using gestational age as the indicator. *(41)* The ICD-11 defines deaths occurring in the perinatal period to include antepartum and intrapartum stillbirths and deaths occurring during the first seven days of life (early neonatal deaths). It is important to note that WHO extends the 'perinatal' period for MPDSR purposes. In the MPDSR process it is possible to review newborn death cases until 28 days after delivery. Thus, MPDSR's perinatal period covers late neonatal deaths.

2.1.2 MPDSR laws, regulations and policies

Effective implementation of MPDSR systems relies on a cohesive relationship between domestic laws, regulations and policies. The nature of these tools and their terminology are country specific. Understanding how the available domestic instruments interact is crucial for creating a robust MPDSR framework. Despite country differences, the following provides a general overview about the scope and differences between laws, regulations, and policies.

MPDSR laws

Laws are generally rules of conduct enforced by a controlling authority, governing the behaviour of individuals, corporations, associations, and government agencies. Typically enacted by the legislative branch and recognized in the constitution, laws relevant to maternal and perinatal health, and reproductive healthcare, span multiple legal sources. These may include the constitution, civil and criminal codes, public health laws, sexual and reproductive health laws, women's and children's health laws, gender equality laws, family law, and social care law.

MPDSR regulations

Regulations may include orders and decrees from presidential, executive, ministerial, and other administrative bodies, as well as quasi-nongovernmental regulations. They can have the force of law but are not ratified by the legislative branch. They can be issued by various authorities, from national to communal levels, and are sometimes called ordinances at the municipal level. The authority of these regulations varies based on the issuing body and context. Regulations often implement or define laws, with the law directing the appropriate authority to act. Some regulations, like those from professional health associations, guide conduct but lack legal force.

MPDSR policies

Governments establish policies through various ministries and levels, outlining a broad range of intentions and activities. Policies are high-level plans, often aimed at specific sectors like health, and may focus issues such as maternal and perinatal health and MPDSR. They can be national or local in scope and generally do not have the force of law, though they may align with legal and regulatory frameworks.

3. Strengthening MPDSR-related laws and regulations

The rationale for codifying a comprehensive MPDSR system within national legal frameworks is multifaceted. Laws and regulations can provide accountability for MPDSR systems. Effectively implementing MPDSR through laws and regulations can contribute to fulfilling states' human rights obligations. Integrating a human rights-based approach into MPDSR requires combining the goals of ending preventable maternal and perinatal deaths and empowering women and newborns by respecting, protecting, and fulfilling their human rights. It also requires that governments ensure the availability, accessibility, acceptability, and quality of maternal, newborn and child healthcare services and facilities while efficiently tackling the underlying causes of the major obstacles faced in attaining the highest standards of health through sustained transformative strategies.

Maternal health is a major human rights concern with respect to access to health services, removing existing inequalities and the addressing the underlying social determinants of women's health. *(42,43,44,45)* Perinatal deaths encompass antepartum and intrapartum stillbirths and deaths occurring during the first seven days of life (early neonatal deaths). Newborns and children are rights holders requiring access to health services and have the right to be registered at birth with a name. *(46)*

The healthy development of a wanted pregnancy and access to maternal services is an important human rights concern for women, newborns and children. The causes of maternal and perinatal death are interrelated, with childbirth being the most critical event. *(47,48,49,50)* Pregnancy, delivery, childbirth and the ante- and postnatal periods are associated with certain health risks that can be managed with preventative care and such care positively impacts the health and development of the foetus and newborns. *(51)* MPDSR systems should promote the human rights of women and newborns, including the rights to life, health, equality and non-discrimination, human dignity, and freedom from torture, cruel, inhuman, or degrading treatment or punishment.

The distinct rights, needs and interests of women and newborns must be comprehensively addressed in MPDSR laws and regulations. This includes government obligations to take appropriate action, which may include the adoption of laws or regulations, to conduct MPDSR and report on their progress in reducing maternal and perinatal mortality. *(52,53)* MPDSR is a quality improvement intervention that not only provides information on the current status of maternal and perinatal mortality but is also a vital tool for informing actions to reduce deaths and fulfil the rights of women and newborns.

MPDSR laws and regulations enable the creation of a coordinated and evidence-based national monitoring and evaluation framework for maternal and perinatal deaths. They can define legally enforceable roles and responsibilities for those involved in the MPDSR process and recognize the rights and duties of the various participants in the system. An MPDSR legal framework can promote accountability, empowerment, participation, equality, and transparency through the establishment of MPDSR committees accountable for implementation, define regularly scheduled parliamentary reviews for monitoring, guarantee appropriate budgetary allocation, and enable the participation of affected communities.

MPDSR laws and regulations can also facilitate a no-blame approach for MPDSR. MPDSR systems must ensure the privacy, confidentiality, anonymity, and autonomy of those participating in the MPDSR process, including data and information confidentiality. MPDSR information and data must remain separate from litigation. Access to justice, remedies, and redress for maternal and perinatal deaths should be guaranteed in distinct mechanisms separate from MPDSR. MPDSR laws and regulations can advise on the existence of separate remedial and redress mechanisms for families and communities.

The rationale, goals, and objectives for strengthening MPDSR laws and regulations are summarized below. MPDSR laws and regulations:

- create legal accountability;
- contribute to fulfilling the human rights of women, newborns, and affected communities;
- inform action to eliminate preventable maternal and perinatal mortality;
- enhance MPDSR participant safeguards; and
- advise on separate remedial and redress mechanisms for families and communities, as well as independent disciplinary and liability mechanisms for healthcare workers.

3.1 Stakeholder involvement in the MPDSR legal and regulatory process

It also is crucial to involve all relevant stakeholders in any MPDSR reform process. Each stakeholder has a distinct role to play in the development of MPDSR laws, regulations, and policies. A non-exhaustive list of key stakeholders and their potential roles in MPDSR development are outlined in Table 1 below.

Table 1: Key MPDSR stakeholders

Stakeholder category	Specific stakeholders	Role in MPDSR development
Legislature	Parliamentarians, Parliamentary Bodies, and Other Legislators	Budgetary oversight and lawmaking
Government agencies and policymakers	Ministry of Health, Education, Justice, Gender, Women's Health and Children's Health, Local and Regional Health Authorities, Social Affairs, Finance	Provide regulatory and policy direction, including sustained prevention strategies, implementation, and monitoring
Healthcare providers	Healthcare Professionals (Doctors, Nurses, Midwives), Hospital and Clinic Administrators	Report, review, and respond to maternal and perinatal deaths, and implement protocols
Public health organizations	Public Health Institutes, Institutions, Professional Organizations and Research Organizations	Provide standards, ethical guidelines, research support, advocacy, and community-based support
Civil society and community representatives	Civil society organizations, Community Based Organizations, and Advocacy Organizations, Affected communities	Advocating for community needs and fostering community engagement
Legal and human rights entities	Legal Experts, Human Rights Organizations, Treaty Monitoring Bodies	Monitor compliance with legal standards and advocate for human rights
International and regional organizations	WHO, United Nations Agencies (e.g., United Nations Children's Fund, United Nations Population Fund), and Regional Bodies (e.g., European Union, African Union Commission, Organization of American States, The Association of Southeast Asian Nations)	Offer technical guidance, best practices, funding, and expertise

Stakeholder category	Specific stakeholders	Role in MPDSR development
Academic and training institutions	Universities, Medical Schools, other relevant training institutions	Contribute to transformative strategies promoting a culture of equality, capacity building, education, and training for stakeholders
Technology and data management experts	Health Information System Developers, Data Analysts and Epidemiologists	Design data systems, analyse data to identify trends and effective interventions
Funding agencies and donors	Private Foundations	Provide financial resources for implementation and sustainability
Media and public relations	Media Organizations	Raise awareness and educate the public about the underlying causes rooted in the socio-economic determinants of maternal and perinatal health issues

Participation and engagement from key stakeholders are crucial for addressing gaps in MPDSR implementation. Involving a diverse range of stakeholders helps to ensure that the MPDSR framework comprehensively addresses all aspects of maternal and perinatal health, from clinical care to community engagement. For instance, the Ministry of Health should be engaged throughout the legal and regulatory reform process helps to ensure the development of comprehensive MPDSR frameworks.

WHO's technical guidance identifies that the following principles should be applied to achieve an accountable and participatory MPDSR system: *(54)*

- **Intensive and inclusive planning:** Establish a comprehensive code of conduct and favourable legal environment for setting up an MPDSR system; establish clear standards for conducting death reviews; engage and educate all stakeholders, including communities and the private sector; engage relevant professional associations; and identify key champions.
- **Sustained collective learning, for action at all levels:** Promote shared responsibility and teamwork; integrate MPDSR principles and guidance in training curricula; and encourage collective learning for action at different levels, from the community to the healthcare delivery system.
- **Optimize opportunities for achieving wider benefits:** Develop a culture of accountability, equality, and quality of care, centred on the continuous improvement of healthcare and services; improve recordkeeping, data flow, data quality, and health management information systems; strengthen existing systems, including vital registration and population/pregnancy surveillance; and focus on enhancing the understanding of the burden and level of maternal and perinatal deaths in the population.

3.2 Public health objectives

MPDSR integrates two systems for death surveillance and review, one for maternal deaths and the other for perinatal deaths. WHO has published technical guidance for maternal deaths in *Maternal death surveillance and response (MPDSR): technical guidance information for action to prevent maternal death* (2013), *(55)* and for perinatal deaths in *Making every baby count: audit and review of stillbirths and neonatal deaths* (2016). *(56)* In 2021, the WHO established a comprehensive and unified MPDSR framework in *Maternal and Perinatal Death Surveillance and Response: Materials to Support Implementation* (2021). *(57)* These guidelines set out the public health objectives for MPDSR.

MPDSR has two underlying purposes. *(58)* First, MPDSR provides information about modifiable factors that lead to preventable maternal and perinatal deaths. This information can be used to guide quality improvements critical for preventing similar deaths in the future within the healthcare system and guide actions at the community level, and at the intersectoral level (i.e., in other governmental and social sectors). Second, MPDSR establishes a framework for accurately quantifying maternal and perinatal mortality levels, enabling a more precise assessment of the effectiveness of interventions aimed at reducing these deaths. MPDSR also helps provide accountability for results and compels decision-makers to give such deaths the attention and response they deserve.

The information provided by MPDSR can: increase awareness of maternal and perinatal mortality at the community, healthcare system, and intersectoral levels; lead to changes in the practices of the public, communities, and healthcare providers; and increase resources for activities that prevent maternal and perinatal deaths. MPDSR requires an enabling environment – one that promotes collaboration over blame – to effectively utilize its findings and translate them into action. The system should enhance and build upon existing maternal and perinatal death information systems.

The smooth and accountable operation of the MPDSR system is more likely when supported by enabling regulations and legal protections. The regulations and legal protections should include provisions that advance the goals of MPDSR such as: mandatory reporting; mandates for the establishment of the MPDSR system, including a national MPDSR steering committee and dissemination of results to state entities, civil society, professional organizations, Nongovernmental organization (NGOs), donors, and other relevant stakeholders; establishment of clear monitoring and evaluation systems; guaranteed protection for healthcare workers and individuals who participate as reviewers or respondents against legal action within the scope of the MPDSR process, and ensuring confidentiality while protecting access to justice for the community.

3.2.1 Civil Registration and Vital Statistics (CRVS) standards developed by United Nations agencies

MPDSR processes are closely intertwined with CRVS systems. A well-functioning CRVS system is essential for MPDSR, as it provides the necessary infrastructure for collecting, compiling, and reporting data and statistics related to maternal and perinatal death. The United Nations Department of Economic and Social Affairs (UN DESA) has developed principles and guidelines for the civil registration of vital statistics in Principles and Recommendations for a Vital Statistics System. *(59)* UN DESA has reported that national CRVS systems in many low-income countries have tended to neglect requiring registration of perinatal deaths and highlights the value of such registration for measuring perinatal mortality and pregnancy outcomes. Moreover, the registration of perinatal deaths is essential for building a fair and equitable health system that addresses structural inequalities. Perinatal death statistics are linked with the demographic characteristics of the would-be mother.

Significantly, UN DESA underscores that CRVS systems play a crucial role in the realization of human rights. In particular,

> human rights that require States to ensure that vital events are registered, e.g., the right to be registered and named, pursuant to article 7 of the Convention on the Rights of the Child and (b) human rights whose exercise may depend on the vital events, having been registered, for example, the right to vote, pursuant to article 21 (3) of the Universal Declaration of Human Rights. *(60)*

Implementation of CRVS systems can also be linked to state obligations to respect, protect, and fulfil the right to health as this data informs the delivery and quality of health care services. *(61)*

CRVS systems should be established and enforced through a legal framework that is guided by the following principles.

- **Universal coverage and equality:** CRVS systems should cover all vital events and include all geographical areas and population groups, without discrimination.
- **Continuity:** Data and statistics must be collected and compiled at regular intervals (e.g., monthly or quarterly) and followed by annual reporting.
- **Confidentiality must be protected:** Data should only be used for statistical purposes.
- **Regular dissemination:** At a minimum, CRVS should achieve two goals. First, the statistics should be analysed and disseminated to stakeholders monthly or quarterly to facilitate timely health interventions. Second, there must be dissemination of detailed annual tabulations of each type of vital event, cross-classified by its demographic and socioeconomic characteristics.

The above principles complement the minimum normative standards for a legal framework for national MPDSR, discussed in Section **5 Elements of MPDSR laws and regulations** below.

3.3 Human rights obligations, standards, and guidance

A number of human rights bodies at the international, regional, and national levels have developed clarifications that may assist in the implementation of human rights obligations that are relevant to the establishment of MPDSR systems and quality of care improvements. Implementation of obligations for the protection of maternal and newborn health, for example, can promote the effective implementation of MPDSR systems. These human rights standards contribute to the effective implementation of MPDSR systems, ensuring that maternal and newborn health are prioritized. States that are parties to relevant international treaties have an obligation to uphold and ensure the rights contained within those treaties. Those states that have signed but not yet ratified an instrument are bound to ensure that nothing is done which would defeat the object and purpose of the treaty, pending a decision on ratification. States may also face regional and national obligations that build on these international human rights commitments. The following provides an overview of key human rights standards and guidance relevant for MPDSR.

3.3.1 International human rights standards

Human rights treaty bodies

General comments and general recommendations

Several human rights standards aimed at improving maternal and newborn health outcomes provide a supportive framework upon which to build effective MPDSR systems. Human rights related to maternal and newborn health include the rights to life, health, equality and non-discrimination, human dignity, and freedom from torture, cruel, inhuman, or degrading treatment or punishment.

The International Covenant on Civil and Political Rights (ICCPR), the International Covenant on Economic, Social and Cultural Rights (ICESCR), the Convention on the Elimination of All Forms of Discrimination Against Women (CEDAW), and the Convention on the Rights of the Child (CRC) all include provisions relevant to the development of effective MPDSR systems. Each of these treaties are subject to monitoring conducted by a treaty body. The General Comments or General Recommendations issued by a treaty body clarify the substantive treaty obligations for States parties to the relevant treaty.

General comments and recommendations from the Human Rights Committee, the Committee on Economic, Social, and Cultural Rights (CESCR), the Committee on the Elimination of Discrimination against Women (CEDAW), and the Committee on the Rights of the Child (CRC) emphasize that fulfilment of the right to health, including the right to sexual and reproductive health requires the urgent need for comprehensive healthcare services tailored to women and children, addressing both preventive and remedial measures to reduce maternal and perinatal mortality. The following provides an overview of general comments and general recommendations with respect to the treaty provisions that address issues most relevant to MPDSR. These standards can also assist states that are not parties to these treaties in developing effective MPDSR systems.

Human Rights Committee

Article 6(1) of the ICCPR guarantees the inherent right to life, stating that "[e]very human being has the inherent right to life. This right shall be protected by law. No one shall be arbitrarily deprived of their life." *(62)* The Human Rights Committee, in **General Comment 36**, clarifies the obligations under the right to life, which include ensuring access to essential goods and services, as well as comprehensive sexual and reproductive health education and information. *(63)*

Committee on Economic, Social, and Cultural Rights (CESCR)

The CESCR's **General Comment 14** clarifies the substantive obligations relate to the right to health under **Article 12** of the ICESCR and addresses the importance of quality maternal and child healthcare. *(64)* The Committee stresses the importance of measures to reduce stillbirth and infant mortality rates, including access to comprehensive sexual and reproductive health services (para 14). It advocates for a comprehensive national strategy to promote women's right to health across their lifespan, encompassing treatment and access to a full range of affordable, high-quality healthcare services (para 21). The CESCR calls for addressing the health risks women face, particularly reducing maternal mortality rates (para 21). It further emphasizes the importance of providing essential health services for children, including pre- and post-natal care for mothers (para 22). The Committee emphasizes that the health of adolescents is closely linked to the provision of appropriate healthcare services, including sexual and reproductive health services, which must be delivered with full respect for confidentiality and privacy (para 23). The Committee reinforces the need to ensure reproductive, maternal, and child healthcare (para 44(a)) and categorizes the failure to reduce maternal and infant mortality rates as a violation of the right to health (para 52). The CESCR identifies that the right to health includes the right to confidentiality of personal health data (para 12(b)). It also highlights that 'when formulating and implementing their right to health national strategies, States parties should avail themselves of technical assistance and cooperation of WHO' (para 63).

CESCR's **General Comment 22** on the right to sexual and reproductive health further clarifies the right to health. The Committee calls for the repeal of discriminatory laws and the removal of barriers that interfere with women's access to comprehensive sexual and reproductive health services (para 28). It emphasizes the need for emergency obstetric care and skilled birth attendance, especially in rural and remote areas (para 28). The CESCR further stresses the importance of ensuring universal access to a full range of quality sexual and reproductive healthcare services without discrimination (para 45).

Committee on the Rights of the Child (CRC Committee)

The right to health is contained in **Article 24** of the CRC and its full text can be found in Box 1. Article 24 recognize the right of the child to the enjoyment of the highest attainable standard of health and ensure no child is deprived of access to health services. States have treaty obligations to reduce perinatal mortality and ensure appropriate maternal care and eliminate harmful traditional practices.

Box 1: Article 24 of the Convention on the Rights of the Child

Article 24 of the Convention on the Rights of the Child

1. States Parties recognize the right of the child to the enjoyment of the highest attainable standard of health and to facilities for the treatment of illness and rehabilitation of health. States Parties shall strive to ensure that no child is deprived of his or her right of access to such health care services.

2. States Parties shall pursue full implementation of this right and, in particular, shall take appropriate measures:

(a) To diminish infant and child mortality;

(b) To ensure the provision of necessary medical assistance and health care to all children with emphasis on the development of primary health care;

(c) To combat disease and malnutrition, including within the framework of primary health care, through, inter alia, the application of readily available technology and through the provision of adequate nutritious foods and clean drinking-water, taking into consideration the dangers and risks of environmental pollution;

(d) To ensure appropriate pre-natal and post-natal health care for mothers;

(e) To ensure that all segments of society, in particular parents and children, are informed, have access to education and are supported in the use of basic knowledge of child health and nutrition, the advantages of breastfeeding, hygiene and environmental sanitation and the prevention of accidents;

(f) To develop preventive health care, guidance for parents and family planning education and services.

3. States Parties shall take all effective and appropriate measures with a view to abolishing traditional practices prejudicial to the health of children.

4. States Parties undertake to promote and encourage international co-operation with a view to achieving progressively the full realization of the right recognized in the present article. In this regard, particular account shall be taken of the needs of developing countries.

The CRC Committee's **General Comment 15** on the right of the child to the enjoyment of the highest attainable standard of health clarifies the substantive obligations under the right to health. The CRC committee emphasizes that political commitment and sufficient resource allocation are crucial to prevent mortality and morbidity among children (para 1), including the often under-prioritized issue of adolescent morbidity and mortality (para 33).

The Committee highlights the significant impact of the mother's health on child health outcomes, particularly noting the risks associated with poor maternal health during pregnancy and the post-natal period (para 18). Article 24(a) of the CRC calls for state parties to reduce infant and child mortality, the Committee says that this includes neonatal mortality (para 33). The Committee recommends a range of interventions to address various health issues affecting children, including maternal and perinatal mortality audits for accountability (para 34). The Committee underscores that preventable maternal mortality and morbidity constitute grave human rights violations affecting both women and children (para 51).

It stresses the importance of collecting relevant and reliable data to inform health policies and services for children (para 109). It further advocates for robust monitoring and evaluation mechanisms, using disaggregated data to redesign and improve health policies and services (para 117). The Committee also calls for national accountability mechanisms to monitor, review, and act on health data to improve children's health outcomes, ensuring transparency and regular evaluations (para 118).

Committee on the Elimination of Discrimination against Women (CEDAW Committee)

The CEDAW Committee's General Recommendation 24 on the right to health outlines states' obligations under **Article 12** of the CEDAW, focusing on key issues in women's healthcare. The Committee underscores the alarming rates of maternal mortality and morbidity worldwide, suggesting that these figures indicate potential breaches by States in ensuring women's access to healthcare (para 17). The Committee calls for detailed state reporting, required under Article 18 of CEDAW, on the measures taken to provide appropriate services during pregnancy, childbirth, and the post-natal period, specifically noting the need to include data on the impact of these measures on reducing maternal mortality and morbidity, particularly among vulnerable populations (para 26). Furthermore, the Committee emphasizes the state's duty to provide free services where necessary to ensure safe pregnancies and childbirth (para 27). General Recommendation 24 points out that many women suffer or die from pregnancy-related causes due to a lack of financial resources to access essential services, highlighting the need for states to allocate maximum resources to ensure safe motherhood and emergency obstetric services (para 27). The removal of socio-cultural discriminatory barriers is essential to addressing maternal and perinatal mortality. The Committee also highlights state obligations to establish an enabling environment through laws, regulations, and policies that remove socio-cultural discriminatory barriers, which negatively affect health care services and hinder the fulfillment of the right to health (paras 14, 31).

Various General Recommendations from the CEDAW Committee address the broader enabling environment necessary to comprehensively tackle the factors contributing to maternal and perinatal mortality. Awareness of these factors may aid in the development of MPDSR committee recommendations to improve quality of care and reduce maternal and perinatal deaths. **General Recommendation 35** *(65)* addresses the elimination of gender-based violence against women, noting that factors like ethnicity, socioeconomic status, and gender identity can exacerbate discrimination (para 12). **General Recommendation 2** *(66)* outlines states' obligations under **Article 2** of CEDAW to eliminate discriminatory laws, embed gender equality in domestic law, and provide access to remedies (paras 31, 36). **General Recommendation 36** *(67)* expands on the right to education under **Article 10** and its link with other rights, including **Article 5**, urging states to eliminate gender stereotyping in education that perpetuates discrimination against girls and women. *(68)* **General Recommendation 21**, *(69)* on equality in marriage and family relations, analyzes **Articles 9, 15, and 16** regarding women's status in family relations, affirming their right to decide on the number and spacing of their children, which affects their access to education, employment, and their physical and mental health (para 21). Together, these General Recommendations can guide states in creating an enabling environment that removes socio-cultural discriminatory barriers, ensuring the fulfillment of the right to health and related rights.

CEDAW Committee - Individual communication procedure

In response to an individual communication, the CEDAW Committee expressed the view that states are responsible for addressing and reducing maternal mortality. The Optional Protocol to the CEDAW provides a mechanism for individuals or groups of individuals to submit petitions concerning rights violations by states that have ratified the optional protocol. *(70,71)*

In the *Alyne da Silva Pimentel v Brazil* decision, the Committee emphasized that obligations under CEDAW require state parties to prevent avoidable maternal deaths and ensure appropriate and non-discriminatory maternal care. Specifically, the Committee identified that the rights to health (Article 12(2)), non-discrimination (Article 1), access to justice (Article 2(c)), and oversight of private health service providers to ensure their compliance with human rights (Article 2(e)), necessitate the following actions:

- guarantee a woman's right to safe motherhood;
- provide non-discriminatory health care that meets the distinct needs of women, including vulnerable and marginalized groups;
- ensure adequate training for health care workers in the context of maternal care and impose sanctions on health care workers for rights violations;
- ensure the quality of care and application of maternal care standards in health facilities in accordance with national and international standards on reproductive health care;
- provide remedies for rights violations; and
- establish maternal mortality committees to reduce preventable maternal deaths. *(72)*

The *Alyne* case has been identified as significant because it applies human rights law to the necessity of implementing a national plan for preventing maternal deaths and highlights the role of Maternal Mortality Review Committees. *(73)* In addition to ratifying CEDAW and other human rights treaties, many states enshrine the rights to health, non-discrimination, access to justice, and oversight of private health service providers in their national constitutions to ensure compliance with human rights. This further reinforces the need to adopt appropriate measures to improve quality of care and reduce preventable maternal and perinatal deaths due to their close link.

Summary

In summary, treaty body recommendations regarding the right to health call for an integrated approach to healthcare that addresses the needs of women and children. They emphasize the importance of political commitment, adequate resource allocation, removal of socio-cultural discriminatory barriers, comprehensive data collection, and robust accountability mechanisms to ensure the realization of health rights.

Human Rights Council and the Office of the High Commissioner for Human Rights

United Nations Human Rights Council resolutions: preventable maternal mortality and morbidity and human rights

The United Nations Human Rights Council has various adopted resolutions, without a vote, on preventable maternal mortality and morbidity and human rights. *(74,75,76,77,78,79,80,81,82)* The Council has urged all states to renew their political commitment to eliminate preventable maternal mortality and morbidity at the local, national, regional, and international levels, to strengthen their efforts to address multiple and intersecting inequalities, and to allocate domestic resources to health systems and the provision of the necessary information and services in relation to the right to the highest attainable standard of physical and mental health, including the sexual and reproductive health of women and girls.

Based on these resolutions, the Office of the United Nations High Commissioner for Human Rights has produced technical guidance on the application of a human rights-based approach to the implementation of policies and programmes to reduce maternal mortality and morbidity. *(83)*

This guidance specifically calls for the routine review of all maternal deaths, in order for lessons to be learned at all levels of the health system: from individuals' behaviour and practices to national policies, and throughout the continuum of care from home to hospital. The guidance specifies that maternal death reviews should be conducted in the spirit of shared responsibility, rather than attributing blame, in order to inform action to prevent future maternal deaths. *(84,85)* At the request of the Human Rights Council, five biennial follow-up implementation reports were produced since 2014. *(86,87,88,89,90)*

Recognizing the disproportionate inequalities faced by women in relation to pregnancy, childbirth, and sexual and reproductive health, the Council adopted a resolution in 2018, without a vote, titled "Preventable maternal mortality and morbidity and human rights in humanitarian settings". This resolution calls for urgent action by states to ensure accountability and gender-sensitivity, and to provide prompt and effective remedies for the violation of women's and girls' rights related to maternal mortality and morbidity in humanitarian settings. It emphasizes the importance of including transparent forms of monitoring, review, and oversight of humanitarian programmes and policies. *(91)*

United Nations Human Rights Council resolutions: preventable mortality and morbidity of children under 5 years of age

The Human Rights Council's resolution on preventable mortality and morbidity of children under 5 years of age, adopted without a vote, is applicable to newborns. *(92)* It calls on states to adopt a human rights-based approach to reducing and eliminating preventable child mortality and morbidity. It highlights the importance of scaling up child healthcare services and adhering to human rights principles, including the principles of equality and non-discrimination, participation, transparency, the best interest of the child, and accountability. It also recognizes that mortality and morbidity are highest amongst poor and marginalized communities.

The Office of the High Commissioner for Human Rights has issued technical guidance on the implementation of policies and programmes to reduce and eliminate preventable mortality and morbidity of children under 5 years of age. *(93)* The guidance reiterates the imperative of adopting a human rights-based approach. It highlights that several key treaties are particularly relevant to addressing child mortality and morbidity, including the CRC, the ICCPR, the ICESCR, and the CEDAW.

The technical guidance provided should inform the establishment of MPDSR systems. It describes surveillance of and response to child mortality as a positive obligation on states, essential for fulfilling the human rights of children, especially their rights to life and health. In order to fulfil their human rights obligations, states should establish a framework for regular monitoring of child mortality and morbidity, identifying causative factors, responding promptly to these causes, and monitoring and evaluating the response through accountability mechanisms. This includes the following:

- monitoring and evaluating data relating to child mortality and morbidity, collected regularly and rigorously to track the realization of human rights and inform policies, programmes, and services;
- establishing functioning health information and surveillance systems that ensure the reliability of data, are transparent and consistent, while protecting the right to privacy of children and their families;
- disaggregating data to reveal which groups of children are being left behind. This data should be regularly used to track progress and direct action;
- ensuring accountability mechanisms for monitoring and review, with recommendations for timely remedial action based on the findings;
- using timely and accurate information relating to all factors affecting child mortality and morbidity to improve child health;

- including civil society and other community members in the collection and analysis of child health data for monitoring and evaluation, and making decisions arising based on data analyses; and
- ensuring that affected populations and other stakeholders participate in efforts to address child mortality and morbidity, for example, by consulting parents and caregivers on their priorities for addressing child mortality and morbidity, and including community members in health facility management committees.

The technical guidance on child mortality and morbidity '[u]rges States to disseminate the technical guidance and to apply it, as appropriate, in the design, implementation, evaluation and monitoring of laws, policies, programmes, budgets and mechanisms for remedy and redress aimed at eliminating preventable mortality and morbidity of children under 5 years of age'. *(94)*

3.3.2 Regional human rights obligations, standards, and guidance

At the regional level, the Inter-American Court of Human Rights issued a decision in 2009 concerning the preventable maternal death of an Indigenous woman in the case of the Xákmok Kásek Indigenous Community v Paraguay. *(95)* This case involved violations of the rights to life, equality and non-discrimination due to the quality of maternal care received. The court identified that the American Convention on Human Rights requires that states implement healthcare policies aimed at preventing maternal deaths. This includes ensuring the training of skilled birth attendants, providing high-quality pregnancy-related care, and documenting maternal deaths. *(96)*

The Inter-American Court of Human Rights emphasizes the importance of integrating equality, non-discrimination, and community participation into health systems. Community involvement can help prevent discrimination and ensure equality by facilitating demographic representation, particularly in contexts where social and economic disparities affect reproductive health. Decisions about the design and implementation of a healthcare system should involve the affected population. This is an important consideration throughout the MPDSR legislative process and in the design of MPDSR committees.

The European Convention on Human Rights also guarantees several fundamental rights, including the right to life, the prohibition of torture, respect for private and family life, and the right to non-discrimination, among other rights, to ensure access to quality health care for everyone. *(97)* It also addresses the specific needs of marginalized groups. Individuals or families who experience a rights violation can submit an individual complaint to the European Court of Human Rights after exhausting all domestic remedies. The European Court of Human Rights has examined various issues related to reproductive rights and children's rights. *(98)* Similarly, other regional human rights bodies, including the African Commission on Human and Peoples' Rights, responsible for interpreting the African Charter on Human and Peoples' Rights, have the authority to assess rights violations that are relevant to the development of MPDSR systems.

The Protocol to the African Charter on Human and Peoples' Rights on the Rights of Women in Africa, known as the Maputo Protocol, addresses various rights that contribute to advancing MPDSR frameworks. Adopted in 2003, the protocol mandates state parties to ensure comprehensive health rights for women, including the right to health and reproductive rights outlined in Article 14, which covers the provision of maternal health care services. Article 2 focuses on the elimination of discrimination against women, while Article 4 addresses the rights to life, integrity, and security of the person, including freedom from exploitation and cruel, inhuman, or degrading treatment. The Maputo Protocol creates an environment that supports the establishment and operation of MPDSR frameworks by emphasizing the elimination of practices harmful to women's health and promoting access to quality health services. The rights contained within the protocol underscore the need for state interventions to fulfil these obligations which are highly relevant to effective MPDSR systems, which may include systematic monitoring and response mechanisms to address maternal and perinatal deaths.

3.3.3 National human rights obligations, standards, and guidance

Across various countries, courts have increasingly recognized that preventable maternal deaths constitute violations of fundamental rights, including the right to life and health. Legal rulings have held governments and healthcare providers accountable for failing to provide adequate maternal care, often resulting in measures to improve healthcare services, compensate affected families, and ensure systemic reforms. These national legal interventions underscore the importance of legal accountability in protecting maternal health and highlight the potential role of courts in driving improvements in healthcare standards and the development of effective MPDSR systems.

At the national level, India has addressed cases of maternal deaths through its legal system. In 2008, the High Court of Madhya Pradesh held that the inability of women to survive pregnancy and childbirth violates their fundamental rights guaranteed under Article 21 of the Constitution of India. It emphasized that "it is the primary duty of the government to ensure that every woman survives pregnancy and childbirth, for that, and the State of Madhya Pradesh is under obligation to secure that". *(99,100)* In 2010, Delhi's High Court in the *Laxmi* case also relied on the right to life guaranteed in the Indian Constitution to find public health service providers liable for the death of women who were not provided with appropriate healthcare during pregnancy and after delivery. *(101)* Indian courts have determined that the right to health is integral to the right to life. In *Laxmi*, the High Court ordered the state to financially compensate the women's families. They also directed the state to remedy deficiencies in and improve monitoring of public health programmes in order to prevent future deaths. The case highlights that preventable maternal deaths are a human rights matter and central for political, legal and health system accountability, with the support of key stakeholders.

Ugandan courts have also been asked to rule on state accountability for maternal death and morbidity. In 2020, in *Petition 16*, the Constitutional Court of Uganda used the right to health guaranteed by the Constitution to find state healthcare providers liable for the deaths of women in labour due to the lack of appropriate healthcare. *(102)* The court found that their deaths resulted from the failure to provide emergency obstetric care. In addition to awarding monetary compensation to the victims' families, the court also mandated the state to allocate prioritized budgetary resources for maternal health care within specified timeframes. This includes provisions for appropriate staff training at health centres, adequate medical equipment, and ongoing monitoring and evaluation of the mandated interventions. *(103)*

4. Incorporating MPDSR laws and regulations into existing frameworks

The development of an MPDSR framework requires an assessment of surrounding instruments that have a bearing on establishing laws and regulations for MPDSR.

4.1 Assessment of existing laws, regulations, and policies

Laws and regulations to regulate MPDSR should be developed, not in isolation, but alongside other laws, regulations, and policies affecting the health sector, other sectors and the wider legal system. It is therefore necessary to consider all laws, regulations, and policies relevant to MPDSR. These might include a constitution, civil code, gender equality law, health law, sexual and reproductive health law, administrative law, common law, customary law, and decisions by the constitutional or supreme court or any lower court. All of these may have federal or subnational application.

MPDSR systems are implemented within the context of existing laws that protect the rights of women and newborns. These laws allow individuals to pursue legal proceedings for issues such as negligence or breach of privacy. Therefore, any new MPDSR laws or regulations must consider the broader legal framework, including laws related to legal and criminal liability and data protection.

Furthermore, an existing MPDSR system, or components of such a system, may be governed by regulations, such as ministerial orders, or by policy alone. Ensuring that new laws or regulations complement any pre-existing regulatory or policy system is an essential step in the MPDSR legal and regulatory process.

Additionally, it is crucial to assess any existing laws or regulations that mandate the reporting of maternal and perinatal deaths. This includes evaluating whether the country has laws or regulations that impose a legal duty to review maternal and perinatal deaths and establish an institutional framework for implementing maternal and perinatal death surveillance and review.

4.2 Assessment of laws and regulations requiring the reporting of maternal deaths

While MPDSR involves both maternal and perinatal deaths, it is useful to discuss the reporting requirements for maternal and perinatal deaths separately, as they have important legal differences. This section discusses the reporting requirements for maternal deaths.

Challenges in measuring maternal mortality still exist, including deaths related to unsafe abortion, where underreporting and misclassification are significant barriers. (*104*, *105*) The efficacy of MPDSR depends crucially on the timely provision of information about maternal and perinatal deaths and a prompt review of their causes and circumstances. Establishing a legal or regulatory mandate for reporting maternal deaths is vital for enabling a state to identify and take actions to prevent future deaths. Mandatory reporting promotes accountability and transparency by revealing the true scale of maternal deaths, which may be underreported in death certificates. It involves a clear chain of institutional responsibilities for reporting and facilitates the "response" component of MPDSR, enabling timely interventions to address modifiable factors at facilities and within the community. MPDSR is a quality improvement intervention to identify and analyse maternal and perinatal deaths to guide preventive measures.

4.2.1 Using an existing system of death notification

One way to make maternal deaths notifiable is to integrate maternal death registration into the existing vital registration system. All countries legally require a death certificate, stating the cause of death and signed by a doctor or another authorized person, such as a coroner, as a condition for burial. Consequently, all countries operate or have the basis for operating a vital registration system, although not all achieve full vital registration. Therefore, notifying a maternal death may only require modifying existing forms to require recording the death as a maternal death. For instance, some countries have implemented a legal requirement to report maternal deaths using their pre-existing system of death notifications for the general population.

Historically, some countries have been pioneers in developing national systems for maternal death review, combining policy and existing legal frameworks for reporting deaths to generate systemic insights into maternal mortality. *(106)* This approach has been increasingly adopted worldwide.

Procedurally, in some countries, national health authorities are typically responsible for gathering and publishing data on the causes of death. *(107)* Certifying a death may involve a two-stage process. For example, first, the physician who confirms a death must immediately report it to a designated national or regional agency, such as a tax or civil registry, fulfilling a legal requirement before burial can be authorized. This initial report usually does not include the cause of death. The second stage may involve the submission of a completed death certificate to the relevant health authority, typically within a few weeks, by the patient's usual physician or the last physician who attended to the patient. This certificate may follow a standardized format, such as the International Form of Medical Certificate of Cause of Death, and uses WHO's ICD system to identify specific causes of death, including maternal deaths. The information may then be recorded in a national cause of death register.

Based on the findings from these review processes, health ministries and related agencies may implement various preventive measures to improve quality of care and reduce preventable deaths, such as increasing the number of trained healthcare professionals and strategically placing them within the health system. This systematic approach allows countries to identify and address key factors contributing to mortality.

4.2.2 Mandating maternal death reporting through regulation

Another approach to institutionalizing maternal death reporting is to impose a specific duty to notify. Low- and middle-income countries are increasingly including maternal death on the list of notifiable deaths, which helps capture data that might be missed by the vital registration system. This regulation can typically be achieved through the executive authority of health ministries by invoking administrative public health regulations to add maternal mortality to the existing list of notifiable conditions.

In some countries, health facilities are required to report any maternal, perinatal, neonatal, or child death within 24 hours to relevant district authorities and the civil registration system for the issuance of a death certificate. *(108)* Maternal deaths are also reported to the coordinator of maternal, newborn, and child health at a higher administrative level. Deaths must be recorded and reported within 24 hours and reviewed within 48 hours, supporting the concept of "real-time death reporting." *(109)* This approach aims to enable prompt action to address modifiable factors, such as improving skills, addressing staffing gaps, providing necessary resources, or enhancing community education. *(110)*

In other countries, the Ministry of Health has identified underreporting of maternal deaths within health and civil registration systems, despite legal requirements for death certificates. *(111)* To address this, public health regulations have been developed to make maternal deaths notifiable, integrating these notifications into existing health information and surveillance systems. These regulations are supported by guidelines based on WHO technical guidance, highlighting the importance of timely and accountable reporting, including "zero reporting" when no deaths occur.

This strategy aims to enhance maternal death surveillance, accountability, and improve overall maternal health outcomes.

Some countries have established national maternal death surveillance and response system that are officially integrated into the public health emergency management framework, designating maternal mortality as a "weekly reportable condition" to health authorities at the community, health facility, district, regional, and national levels. *(112)* Regulations mandate that maternal deaths be reported within 24 hours, using ICD classifications. *(113)* In other countries, maternal death surveillance and response have been incorporated into existing disease surveillance and response systems, requiring that maternal deaths be reported within 48 hours. *(114)*

4.2.3 Obstacles to implementing the notification of maternal deaths

In some countries, despite legal, regulatory, or policy mandates to report maternal deaths, effective surveillance and review are often hindered by health system limitations and other factors. For instance, while laws or regulation may require death reporting, they may not specifically mandate identifying whether a death is maternal. *(115)* In contrast, some countries' MPDSR systems do require this specification as a matter of policy. Additionally, in countries where death certification is the responsibility of doctors, many maternal deaths go unreported because they occur in communities or health facilities primarily staffed by nurses who are not authorized to complete death certificates. *(116,117)* Moreover, burials can proceed without a death certificate, which is often issued late or not at all. *(118)*

Policies for the notification and review of maternal deaths are in place in some countries; however, data on maternal deaths remain limited due to implementation gaps, such as inconsistencies in reporting standards and variations in the frequency and quality of formal maternal death reviews. These challenges have made it difficult to develop systematic interventions to address maternal mortality. Current drafts of MPDSR laws in some countries seek to address these reporting gaps and expand surveillance to include perinatal deaths.

4.3 Assessment of laws and regulations requiring the reporting of perinatal deaths

To understand the reporting requirements for perinatal death, it is useful to recall what constitutes reportable perinatal death for the purposes of MPDSR, as noted in Section **2.1.1 Perinatal death period**. Civil laws governing vital registration systems at the national level typically require the registration of the death of a person: this includes neonatal deaths but often excludes stillbirths. A specific law or regulation may be required to make stillbirths notifiable. It is not clear how many countries have legislated to require the registration of stillbirths.

In some countries, laws require the registration of stillbirths, typically defining a stillborn child as a fetus that meets specific criteria, such as a minimum weight or gestational age. *(119)* For instance, registration laws may mandate reporting stillbirths to a designated national office without the need for separate death registration. A medical certificate of fetal and neonatal death, often based on WHO/ICD guidelines, must be issued. This certificate generally includes details such as the category of death (fetal or neonatal), maternal demographic data (age, previous pregnancies, ethnicity), primary and contributing causes of death, and other relevant circumstances (e.g., unattended delivery or adverse reactions during labor).

Some countries have established unified MPDSR systems through legislation, coordinated by national committees that oversee maternal and perinatal mortality review processes. *(120)* These committees are typically multidisciplinary, including members from fields such as neonatology, obstetrics, nursing, pathology, clinical genetics, epidemiology, and social work, and increasingly involve parents

and community representatives. Such systems have been credited with reducing perinatal mortality, particularly in communities with disproportionate burdens. *(121)*

The responsibilities of these committees generally include establishing and overseeing surveillance and review processes, arranging review meetings, setting ground rules, and identifying appropriate participants. They also oversee the development and dissemination of practice recommendations, production of surveillance reports, and implementation of clinical guidelines for stillbirth and neonatal death care. Committees are tasked with ensuring compliance with legal requirements for death certification, revising certificates when necessary, and supporting staff involved in perinatal death cases.

Committees often report to national health oversight bodies, providing guidance on public health issues and insights into the factors affecting maternal and perinatal health. This oversight helps ensure the accurate issuance of perinatal death certificates, acknowledging that many are completed before all investigative results, such as autopsy and pathology, are available. The accountability and structured approach offered by these committees are crucial in improving maternal and perinatal health outcomes and enhancing the accuracy and effectiveness of death surveillance systems.

In some countries, a national program was developed to serve as the primary tool for recording all perinatal deaths and identifying avoidable factors. *(122)* Initially launched as a research initiative, it evolved into a mandatory, facility-based perinatal audit program coordinated by a national committee on perinatal mortality and morbidity. A review committee was also established to enforce the program's use, ensuring consistent monitoring, auditing, and identification of preventable causes of perinatal deaths across health facilities. *(123)* To support the effective implementation of the national perinatal audit program, policy guidance has been developed, and regional structures have been established for reporting and coordination with the national committee. *(124)* Dedicated regional review committees, each with a regional coordinator who serves on the national committee, ensure a coordinated flow of data and analysis. The program is implemented at the facility level through regular perinatal review meetings. Coordinators at each level oversee implementation, and the committee publishes regular reports with findings and recommendations to improve perinatal health outcomes.

5. Elements of MPDSR laws and regulations

A distinction must be drawn between a legal duty to report maternal and perinatal deaths and a duty to establish institutional structures and frameworks for such reviews. The existence of a legal duty to review maternal and perinatal deaths can provide the foundation for the creation of a legally empowered institutional framework for conducting such reviews. Available literature shows that countries adopt policies that require maternal and perinatal mortality reviews through their ministries of health. Moreover, many countries implement ministerial policies to establish entities specifically tasked with conducting maternal and perinatal death reviews. These include steering or coordinating committees, Maternal Mortality Review Committees (MMRCs) and Fetal and Infant Mortality Review Committees (FIMRCs). Based on a literature review and results from the WHO Global SRMNCAH Policy Survey, only a few countries were identified as having established both a legal and regulatory framework mandating reviews, along with the necessary institutional infrastructure to support them.

Although the effectiveness of MPDSR institutional frameworks varies widely across countries, they all share a common objective: to establish a confidential no-blame approach that is coordinated nationally and implemented regionally and locally. This approach is used to review maternal and perinatal deaths to identify and address underlying issues with the aim of preventing future deaths in health care facilities and communities.

Laws and regulations can enhance the effectiveness of MPDSR. To be responsive to the purpose of MPDSR, at a minimum, a legal and regulatory framework for MPDSR should address the following elements.

5.1 Mandatory death notification

Requiring the reporting of maternal and perinatal deaths by law strengthens accountability by establishing consequences for neglecting this duty. However, legal obligations can also be established through policies. For instance, where MPDSR policies are explicitly or implicitly incorporated into professional codes of conduct, the obligation to report maternal and perinatal deaths can indirectly achieve legal force.

MPDSR systems should establish legal duties to report maternal and perinatal deaths. Additionally, they should create an institutional framework for conducting maternal and perinatal mortality reviews. This framework includes forming review committees endowed with legal authority. The existence of these committees presumes a pre-established duty to report all maternal and perinatal deaths and necessitates that these reviews be mandatory rather than optional. Therefore, laws and regulations not only reinforce the compulsory nature of these duties but also underscores the importance of adherence.

5.2 Establishing legally mandated MPDSR committees

Establishing maternal and perinatal review committees as legal entities creates accountability for carrying out mandated reviews and allows for the regulation of their composition and operations. This includes granting specific powers and legal protections that enhance their ability to effectively fulfil their duties. Some countries have developed a legal framework for MPDSR, which includes laws governing the creation and regulation of MMRCs and FIMRCs.

5.2.1 Building on existing legal and regulatory frameworks

In some countries it may be possible to build on existing legal and regulatory frameworks. Depending on the country context it may be possible to leverage and enhance current structures rather than replacing them entirely. If this approach is taken, an important consideration is to aim to foster collaboration between different levels of government to unify efforts, share resources, and ensure a coordinated response. Standardizing practices across a country can enhance consistency and effectiveness, as it may allow for the uniform implementation of prevention strategies and make it easier to compare outcomes. Funding and support are also critical components; MPDSR frameworks should provide the necessary financial resources to sustain and expand operations, ensuring that ongoing efforts remain viable over time.

Comprehensive data collection and analysis are central to MPDSR frameworks. Leveraging existing data systems, such as national statistics and surveillance programs, provides a foundational base, while expanding to include detailed investigations offers deeper insights beyond basic statistics. Emphasizing evidence-based, action-oriented approaches helps not only identify problems but also drive the implementation of solutions to enhance care quality and accessibility. Engaging communities and stakeholders in these processes ensures that the actions taken are relevant, accepted, and actionable, fostering a collaborative environment focused on achieving practical and impactful outcomes.

Overall, building on existing frameworks involves enhancing current operations, ensuring funding and support, standardizing practices, and engaging communities, all while designing adaptable models that can be effectively scaled and replicated to address specific needs across various contexts.

5.2.2 Transforming a policy-based MPDSR into law

Some countries are moving to integrate their policy-based MPDSR frameworks into their legal frameworks. This shift may have emerged due to implementation gaps at national and sub-national levels. For example, countries that have relied on national policies to guide the establishment and implementation of MPDSR systems have found inconsistent application at sub-national levels. Inadequate implementation has included a narrow focus on facility-based maternal and perinatal deaths, without dedicating attention to deaths that occur in communities. It was also observed that some local-level MPDSR committees have not consistently achieved effective surveillance and consequently have been unable to identify quality of care interventions to reduce deaths due to these gaps.

To address these challenges, legal frameworks have been proposed to enhance MPDSR implementation. Proposed legislation includes provisions for establishing maternal and perinatal death review committees and coordinating structures at both national and sub-national levels. Some proposed laws outline detailed regulations for committee composition, access to medical records, data collection, review processes, confidentiality, legal protections, reporting, and information dissemination.

Disparities have also been observed concerning national MPDSR policy uptake and practical implementation in various countries, with the application reported as sporadic and ad hoc. In some areas, formal reviews of maternal and perinatal deaths are primarily conducted at major hospitals, but the standards of review are not uniform, leading to causes of death not being consistently assigned according to international standards. These gaps have impeded the effective implementation of MPDSR systems.

To address these issues, some countries have proposed national bills, supported by sub-national legislation, to establish a legal basis for MPDSR. Proposed laws aim to improve quality of care and

develop measures to reduce maternal and perinatal deaths in alignment with human rights outlined in domestic constitutions and the Sustainable Development Goals. Some provisions include guidelines on death notifications, the establishment of review committees and coordinating structures, regulations on committee composition, access to records, data collection and review, confidentiality, legal protections for participants, and systematic reporting and information dissemination.

5.2.3 Mandate of the committees

For an MPDSR system to be effective, it is crucial to clearly define the mandate for all participating institutional entities. These committees should not only identify the causes of death and contributory factors but also provide recommendations to relevant authorities aimed at reducing maternal and perinatal deaths. In some countries, specifically regarding maternal death, national legislation specifies this mandate. National legislation has included guidance on the composition of MMRCs, data collection and review, confidentiality and legal protections, reporting, information dissemination, and national funding to support the establishment of MMRCs at the sub-national level that comply with this guidance. FIMRCs have been established on a similar legal footing. In some countries, these aspects are complemented by sub-national-level laws that have implemented these systems.

A clearly defined mandate strengthens the effectiveness of maternal and perinatal review committees by clearly outlining their responsibilities and powers. This includes granting access to confidential medical records and allowing them to contribute to budgetary and other strategic decision-making processes. Such a mandate is essential for empowering these committees to effectively contribute to reducing maternal and perinatal mortality.

5.2.4 Composition of maternal and perinatal death review committees

Globally, review committees under existing MPDSR systems are generally composed exclusively of healthcare professionals from various clinical specialities. However, to ensure a balance of relevant expertise and public accountability, it is recommended that for MPDSR committees include a broader range of participants. This could include other state actors such as representatives from the Ministry of Women's Affairs or the Ministry of Justice, along with non-state actors like members of human rights, women's, or children's rights commissions. Additionally, true community representation can be achieved by incorporating civil society members, including representatives from women's and children's rights NGOs working on maternal and perinatal mortality, as well as representatives from communities directly affected by these issues.

To effectively address social inclusion and community representation, it is important to recognize that maternal and perinatal mortality disproportionately impacts marginalized groups. Therefore, MPDSR committees should adopt a multisectoral membership, including clinical specialists, public health officials, epidemiologists, and statisticians, as well as representatives from communities most affected by pregnancy-related or pregnancy-associated deaths and those with limited access to maternal and perinatal healthcare services. This diverse membership is vital for ensuring the equitable distribution of health resources and achieving redistributive justice across all geographic regions within the area concerned.

Avoiding conflicts of interest is crucial when appointing members for MPDSR review committees. Conflicts can arise when expert committee members are affiliated with a facility involved in a case under review or may participate in disciplinary proceedings related to the same case, though outside the review context. It is important to avoid placing committee members in compromising positions. For example, in some countries, the maternal and perinatal death review system requires that clinicians on the review panel be independent of the hospital where the patient died and that they do not know the identities of the clinicians involved in the patient's care. *(125)*

5.3 Legal and regulatory solutions for a no-blame approach in MPDSR systems

The MPDSR system should adopt a no-blame approach to encourage open and honest participation. Laws and regulations must safeguard the privacy and autonomy of individuals and families involved in an MPDSR inquiry. Concurrently, remedial and redress mechanisms should exist to support the families and communities of women and newborns who die from preventable causes, including preventable stillbirths, as well as independent disciplinary and liability mechanisms for healthcare workers. These may include court proceedings, patient rights mechanisms, or disciplinary actions. Laws and regulations should ensure the separation yet co-existence of these systems, maintaining confidentiality in the review process while also facilitating access to justice for the families affected and accountability for healthcare workers. This dual approach helps protect the rights of women and newborns while promoting accountability and corrective actions.

5.3.1 Privacy and confidentiality, data protection

MPDSR respondents should be fully informed of the purpose of the investigation, and their participation should be voluntary. Informed consent should be obtained prior to participation from healthcare workers, families, community members, and other individuals involved in the process.

The privacy of the deceased—whether a woman, stillborn baby, or neonate—as well as that of the affected family and healthcare workers, must be protected by ensuring confidentiality throughout the MPDSR process. This includes, among other things, ensuring confidentiality for families during MPDSR and verbal autopsies in all cases, including those involving abortion or suicide. Details concerning these individuals should be anonymised in the report and any associated documents. The system for reporting maternal and perinatal deaths should enable confidential reporting by healthcare professionals, healthcare facilities, and individuals responsible for completing death records, such as medical examiners and coroners. It should also allow for voluntary reporting by family members or others. All MPDSR proceedings must remain confidential to prevent the disclosure of identifying information about any specific mortality case. Information from committee proceedings, including deliberations and records, should not be made public unless explicitly authorized by law.

To safeguard privacy and confidentiality, MPDSR laws and regulations can mandate explicit requirements. For instance, some sub-national laws require committee members to sign a confidentiality agreement before accessing any case-related information. *(126)* Additionally, some MPDSR systems stipulate that all notes containing identifiable information collected for audit purposes must be destroyed after a specified period. In other countries, guidelines exist that specify, for example, that all case notes must be destroyed following the publication of annual reports on maternal and perinatal deaths. *(127)* Guidance also specifies that all retained information must be handled in a manner consistent with national data protection and freedom of information laws. Similar rules have also been applied to the management of perinatal data.

MPDSR committees can rely on a checklist to ensure the protection of confidentiality and privacy, based on the following considerations. *(128)*

- MPDSR meetings should be closed to the public, meetings should be confidential and minutes should be secure.
- All review committee members should sign a confidentiality agreement prohibiting them from discussing the case outside the review meeting.
- All abstracted records, including medical and social service records and interviews, should be stored securely with strictly prescribed access and/or passwords.

- All identifiers should be deleted from abstracted records and the interview.
- The case summary should be anonymous, it should not include the names of participants or local service providers.
- The confidentiality of the review should be guaranteed by the MPDSR methodology protocol.
- MPDSR committee members should consider whether the process is also protected by relevant domestic rules, or by any other authority.
- Although confidentiality must be ensured, effective mechanisms must be available for remedies and redress.

Maintaining confidentiality is also an important consideration in protecting participants against legal and disciplinary proceedings based on the data and information collected throughout the MPDSR process. Legal assurances of confidentiality of data and the prohibition of using MPDSR data in legal and disciplinary proceedings are therefore essential for encouraging participation. MPDSR processes involve reviewing medical records and interviewing persons closely connected with the circumstances of the death under review. Evidence confirms that fear of litigation or disciplinary proceedings based on the MPDSR process deters participation in MPDSR, especially among healthcare workers. *(129)*

Maintaining confidentiality is a means of facilitating a no-blame approach and providing protection from legal and disciplinary proceedings based on data gained from the MPDSR process. Certain sub-national laws have provided for explicit immunities in this regard. For example, some law specify that this includes all proceedings and activities of MMRCs, all opinions of committee members which are formed as a result of the committee's proceedings and activities, and all records obtained, created, or maintained by the committee, including written reports and records of interviews or oral statements. These records shall be confidential, and shall not be subject to public inspection, discovery, subpoena or introduction into evidence in any civil, criminal, or other proceedings. *(130)*

5.3.2 Ensuring access to remedies and redress

Guaranteeing explicit legal protections for reviewers and participants ensures the confidentiality of data and the review process, and shields providers from being sued, prosecuted, or subpoenaed within the context of the MPDSR process. However, legal protections provided by an MPDSR system do not prevent legal or disciplinary proceedings relating to the cause and circumstances of the death. Ensuring access to separate remedies, redress, disciplinary, and liability proceedings is essential for legal accountability. This type of law enables individuals, including families, communities, and healthcare workers, to pursue legal proceedings for issues such as negligence. MPDSR laws or regulations are placed within a broader legal framework, including laws related to legal and criminal liability and data protection. Guaranteeing legal protection within the context of MPDSR proceedings does not confer blanket immunity. Rather, it means that any legal proceedings or disciplinary action arising from a maternal or perinatal death must be entirely separate from the MPDSR process. It is therefore important to build safeguards into the MPDSR system to ensure that these processes remain separate, for example, by ensuring that the same person does not participate in maternal and perinatal death reviews while also taking administrative or legal action against individuals involved in clinical care or the MPDSR process.

Alongside maintaining confidentiality, MPDSR systems should reaffirm that families of women and newborns whose deaths arise from negligence or human rights violations have the right to access justice and effective remedies. They should also identify the disciplinary and liability mechanisms available to ensure healthcare worker accountability and enable investigations into issues such as negligence contributing to maternal and perinatal deaths. Within domestic MPDSR frameworks, it is essential to not only establish an effective MPDSR system but also ensure that it remains separate

from litigation and disciplinary procedures. Ensuring confidentiality in the MPDSR process should not obstruct access to justice. Investigations, lawsuits, prosecutions, patient rights investigations, or disciplinary actions related to the death can still be pursued separately and may lead to legal liability, independent of the MPDSR process.

Personal data or information obtained through the MPDSR process should not be used in any remedial, redress, disciplinary, or liability processes outside the scope of the officially published and publicly available MPDSR report. It crucial that MPDSR legal and regulatory frameworks incorporate safeguards to ensure that protections against legal and disciplinary proceedings within the MPDSR context remain separate from any remedial, redress, disciplinary, and liability proceedings that arise independently of MPDSR processes. For instance, it is vital to prevent individuals who participate in a maternal and perinatal death review from engaging in any administrative or legal actions related to the cases they review, whether as prosecutors, judges, investigators, or human rights commissioners. *(131)* This separation helps maintain the integrity and impartiality of the MPDSR process.

The no-blame approach within MPDSR frameworks aims to protect the confidentiality of data while ensuring access to justice and corrective measures. This approach seeks to balance data protection with accountability, creating a safe environment for reporting and reviewing deaths without fear of disciplinary action based on participation in the MPDSR process. However, the lack of specific legal protections for the privacy of healthcare workers and participants can undermine the no-blame approach, leaving room for potential conflicts of interest and blurring of the division between MPDSR processes and separate, remedial, redress, disciplinary, and liability proceedings.

Although policy guidelines often recommend a no-blame approach, they frequently lack the necessary legal backing to fully assure healthcare workers that the system will protect them regarding their participation in the MPDSR process. The absence of comprehensive laws or regulations mandating the establishment of review committees with a no-blame component has led to a lack of trust in the no-blame approach, especially among healthcare workers. Legal and regulatory reform may be necessary to solidify and effectively implement a no-blame approach for MPSDSR, ensuring it is consistently honored and trusted by all involved, while at the same time reaffirming that negligence will not be permitted, and other mechanisms must be established to ensure accountability for healthcare worker's actions.

5.4 Training

Laws and regulations can address potential gaps in expertise necessary for effectively collecting and assessing data within MPDSR systems. They can mandate training to ensure a sufficient number of experts are available for the MPDSR system to function optimally. For instance, some national legislation requires individuals involved in implementing maternal death reviews to identify and facilitate training for accurate identification and reporting of pregnancy-associated and pregnancy-related deaths. Moreover, ensuring that those conducting reviews have the appropriate knowledge to assign causes of death for perinatal cases and to investigate their links with maternal conditions is crucial. Adequate training helps enhance the reliability and effectiveness of the MPDSR process.

5.5 Reporting and dissemination

Regular reporting and dissemination of MPDSR reviews are essential for learning from the process and for accountability. For example, in some countries, national legislation obliges MMRCs to submit an annual report to the national public health agency, including data, findings, recommendations, and the implementation status of the previous year's recommendations. In turn, the national public health agency is responsible for public dissemination.

In some countries, the maternal death surveillance and response system operates through policy guidance and involves a two-stage review process. *(132)* The first stage is a local review at the facility where the death occurred, allowing immediate identification and rectification of avoidable factors. During this stage, the maternal death is reported to a regional health coordinator. The second stage involves an independent assessment by teams of qualified assessors, such as obstetricians, medical officers, midwives, and anesthetists, who evaluate the circumstances of the death, identify causes, and note avoidable factors using a structured form. The gathered information is entered into an electronic data system that aggregates data from all regions, enabling a comprehensive analysis of maternal mortality trends. At the national level, data from the audit system is used to create annual and triennial reports that identify key causes of maternal mortality and propose recommendations based on stakeholder discussions. This reporting and dissemination process enhances accountability, transforms internal inquiries into public discussions, and fosters broader community participation and engagement with the findings and recommendations.

5.6 Oversight

Establishing institutional frameworks for conducting maternal and perinatal death reviews is an exercise of executive authority that requires democratic accountability. Legislatures play a crucial role in ensuring that effective laws, regulations, policies, and programmes are in place to positively impact maternal and perinatal health. *(133)* Accountability to legislatures can be achieved through mechanisms such as regular ministerial reports, appearances before legislative committees, public hearings, commissions, or inquiries and field visits. In countries where legislative powers are devolved, accountability mechanisms should be appropriately decentralized.

Additionally, oversight mechanisms can help facilitate implementation by securing necessary resources, including financial support. For instance, some national legation authorizes the national public health agency to provide both logistical and financial support for MMRCs. Financial support is vital for the effective functioning and sustainability of MPDSR.

Part 2: Checklist

1. Introduction to the checklist

The checklist is designed to assist in evaluating existing MPDSR and maternal, newborn and child health-related laws, regulations and policies, as well as in developing new ones or implementing amendments. This checklist enables an assessment of MPDSR-specific measures, as well as those related to quality improvement interventions, to determine if an enabling environment exists to improve quality of care and reduce preventable deaths. It is essential to clearly specify the level at which any law, regulation, or policy applies, especially in countries with federal governmental structures or decentralized health systems. The checklist can be used at both national and subnational levels as part of participatory stakeholder processes.

The checklist encompasses eight key areas: guiding principles; goals and objectives; establishment of the MPDSR system; legal and regulatory solutions for a no-blame approach; training; dissemination of MPDSR reviews; and oversight. Each area evaluates specific elements, providing a detailed framework for the comprehensive assessment and development of maternal and perinatal health laws, regulations, polices, or amendment processes. This structured approach ensures comprehensive coverage of essential aspects, enhancing the effectiveness and responsiveness of MPDSR systems.

1.1 Existing laws, supreme or high court decisions, draft laws, regulations, and/or policies

Maternal and newborn health protection, the elimination of preventable maternal and newborn deaths and the establishment of MPDSR systems may already be regulated in various laws, regulations, and policies. These might include civil or criminal laws related women's, children's, maternal or perinatal health, maternal and/or newborn death notification requirements, regulatory and administrative laws, and ministerial regulations. MPDSR systems might also be established through policy frameworks. Additionally, constitutional provisions and supreme or other court decisions may contain relevant protections or recommendations for maternal and perinatal health and MPDSR systems.

Before drafting or amending any laws, regulations, or policies for MPDSR, it is essential to evaluate existing constitutional, legal, regulatory, jurisprudential, and policy sources related to MPDSR to identify any gaps or inconsistencies. This assessment can reveal areas of the MPDSR system that are already sufficiently addressed or expose implementation gaps. Prioritizing the harmonization of existing laws, regulations, and policies with any new measures is essential.

1.1.1 Key considerations

For all questions, key provisions shall be provided, including exact citations of existing constitutional and supreme court provisions, laws, regulations, and policies, as well as the draft text of the newly developed measure.

1.1.2 References

For all questions regarding laws, regulations, policies, and plans, references should be detailed.

References should include:

- precise details of the constitutional provision, court decision, law, regulation, policy, or draft law;
- the issuing authority, such as parliament or the Ministry of Health;
- the number of the law, regulation, policy or draft (if any);
- the date when the law, regulation, or policy was issued or came into force, as well as the date and status of any draft laws or regulations; and
- the name, year and authority of any amendments.

1.1.3 Potential sources of information

To effectively assess the existing MPDSR framework at the country level, it is essential to examine and evaluate the legal, regulatory, and policy instruments that support and facilitate MPDSR systems. This section outlines key sources to consider when analyzing the domestic MPDSR framework, including constitutions, highest court decisions, laws, regulations, and policies. Each of these sources provides valuable insights but requires careful scrutiny to identify the specific legal, regulatory, and policy frameworks applicable to MPDSR in different country contexts. To aid in the collection of relevant instruments, the data collection tool below is organized into the following categories.

Constitution

A constitution is generally the fundamental law of a state, typically written, outlining the distribution of power among government branches and levels, and defining rights and duties of citizens and the state. In some countries, constitutional rights are directly enforceable. In many legal systems, constitutional principles are further detailed through laws adopted by the parliament (or equivalent), including those related to maternal, newborn and child health. Constitutions may also establish rights oversight mechanisms, such as ombudspersons, national human rights commissions, and courts, which may be crucial for monitoring and enforcing maternal and parental mortality reduction strategies within a human rights framework. Constitutions are generally public documents available in official repositories of documents and may be accessed via national governmental websites or international constitutional law websites.

Highest court decisions

Decisions of a country's highest court may provide authoritative interpretations of constitutional matters, including the protection and enforcement of the rights and duties under the constitution. The highest court may be a constitutional court, supreme court or high court, whose decisions are legally binding nationwide and not reviewable by lower courts. In legal systems with both constitutional and supreme courts, the constitutional court may be able to overturn a supreme court decision if it has final jurisdiction in constitutional matters. In federal systems, there may be both federal and state supreme courts, with the federal court having jurisdiction over state courts only as far as federal law applies to state law. Decisions of the highest court can be important because they may provide rulings on the interpretation and/or implementation of a law related to MPDSR, or they may directly call for a legal or regulatory action on MPDSR. In civil law jurisdictions generally, supreme court decisions are not necessarily binding on other cases. However, in practice constitutional/supreme/high court decisions usually provide a very strong precedent, with jurisprudential value for both that court and lower courts. Decisions of highest courts are published in legal gazettes, available in legal and jurisprudential inventories and in databases of the respective courts and the Ministry of Justice. Lower court decisions should be noted if they provide an example for the establishment of an MPDSR system that has relevance as good practice.

Law

Laws are generally rules of conduct or action prescribed or formally recognized as binding or enforced by a controlling authority. They may govern the behaviour of various actors, including people, corporations, associations and government agencies. Laws may be adopted or ratified by the legislative branch of government and formally recognized within the constitution. Laws pertaining to maternal and perinatal health, and reproductive healthcare in general, are typically not confined to a single law. The most relevant legal sources for maternal and perinatal health may include the constitution, civil code, criminal law penal codes, public health (care) laws, sexual and reproductive health laws, women's health laws, children's health laws, gender equality laws, family law, and social care law. These laws and statutes are most likely found in legal or parliamentary databases and libraries. Other documents, including policies, strategies, and plans, may be found in databases or repositories managed by ministries of health, social affairs, and justice, among others.

Regulations

Regulations may include presidential, executive, ministerial and other administrative orders or decrees, as well as quasi-nongovernmental regulations. The executive and/or administrative branch of government can issue orders and decrees that have the force of law. Generally, they are different from laws in that they are not adopted or ratified by the legislative branch. Regulations may be issued by a number of authorities, including national, ministerial, sub ministerial, provincial, district and communal. At the municipal level, regulations are sometimes called ordinances.

The weight and authority of orders and decrees can vary widely, depending on the issuing authority, level, scope, duration, and content. When decrees and orders are used to implement or further define the modalities of laws, the law itself typically directs the appropriate authority to take such action. Other regulations may be important to maternal and neonatal health, but lack the force of law. For example, professional associations governing health professions issue rules and regulations, often called deontological or ethical codes, which guide the conduct of their members. These regulations should align with the constitution, laws, and other higher-level regulations.

Official executive, ministerial, and other administrative orders and decrees should be available at the appropriate repository. Local levels of government frequently make their regulations and ordinances available in the official repository of the local government seat. Other sets of quasi-regulations that may be important to MPDSR, but that lack the force of law, may be issued by professional associations, corporations and other institutions and can be obtained directly from those entities.

Policy

Governments may establish policies through ministries and at various levels, to cover a broad range of intentions and activities. A policy can be a high-level, overarching plan that encompasses the general goals and acceptable procedures of a governmental body. Policies can be aimed at specific sectors, such as health, with a focus on advancing of maternal and perinatal health, and specifically MPDSR. They can be national or local in scope. Generally, policies do not have the force of law, though they can align with legal frameworks. However, policies, can also conflict with existing laws. The government ministry, agency, bureau, or other administrative unit responsible for the development of a policy should be able to provide an official copy.

Draft MPDSR laws, regulation, or policies

An assessment of existing laws, regulations, or policies may reveal that draft measures have already been proposed or initiated by the Ministry of Health, the Ministry of Justice, a parliamentary committee or other actors. Evaluating these drafts can provides insights into proposed MPDSR frameworks and potential areas for improvement.

1.2 Gaps, barriers and inconsistencies in existing laws, draft laws, regulations and/or policies

The assessment of existing laws, regulations and policies may reveal inconsistencies and outdated provisions that do not reflect current evidence and/or human rights obligations. This provides an opportunity to update or annul such provisions, ensuring that new measures are based on up-to-date public health standards and human rights obligations, standards and guidance. The assessment may also uncover gaps where existing laws, regulations, and policies lack the necessary elements for the effective establishment and implementation of an MPDSR system. This process helps determine whether existing provisions adequately address the needs and rights of vulnerable populations and evaluates how relevant laws, regulations, or policies address considerations at both the national and subnational levels.

1.3 Other relevant information

In completing this checklist, it is important to note any considerations that may be relevant for the legal, regulatory, or policy development process. This should include any health system considerations or factors relating to political processes (e.g., changes in government or parliament) that are relevant to the development of a new law, regulation, or policy.

1.4 Newly proposed draft laws, regulations, or policies

Newly proposed laws, regulations or policies should address a number of key elements. Section **5 Elements of MPDSR laws and regulations** in Part 1 details these key elements, each of which should be carefully considered before drafting or revising an existing measure.

1.5 Organizing the results collected

The table below serves as a tool to organize the results obtained from applying the checklist.

Question	Existing law/ supreme court decision	Existing regulation	Existing policy	Draft law, policy, or regulation	Gaps, barriers and inconsistencies	Proposed action to strengthen legal accountability
e.g. 2.1.1						
2.1.2						
2.1.3						
2.2.1						
2.2.2						
2.3.1…						

2. Checklist

2.1 Guiding principles for maternal and perinatal mortality reduction

The establishment of monitoring and surveillance systems to improve quality of care and reduce maternal and perinatal mortality is recognized as a component of human rights accountability in international, regional, and national human rights obligations, standards, and guidance, as well as in public health policy. *(134,135)* Constitutions, supreme or high court decisions, laws, regulations, and policies for women's health, maternal and perinatal health, and MPDSR may establish guiding principles for maternal and perinatal mortality reduction. These principles should be based on public health and human rights considerations. Since the protection of maternal and newborn health is a human rights issue, *(136,137,138,139,140)* healthcare systems must adhere to the principles of equality and non-discrimination, as well as privacy and confidentiality. Measures to address maternal and perinatal mortality should be framed within the context of accountability; effective remedies must be provided for those whose rights are violated, as well as disciplinary and liability mechanisms for healthcare workers.

See Part 1 Sections **3.2 Public health objectives** *and* **3.3 Human rights obligations, standards, and guidance** *for further information.*

2.1.1 Does the constitution or any supreme law include any provisions on protecting maternal and newborn/child health, on reducing maternal and newborn/child mortality, and/or protecting women's and children's health and rights?

2.1.2 Are there any supreme court or other court decisions relating to the protection of maternal and newborn/child health, and/or protection of women's and children's health and rights, and/or reduction of maternal and newborn/child mortality, and/or specific provisions on MPDSR?

2.1.3 Are there any regulations, policies, or plans to promote women's and children's health, and/or reduce maternal mortality, and/or relating to MPDSR in particular? If so, what are the key principles of those regulations, policies or plans?

2.2 Goals and objectives of MPDSR laws, regulations, and policies

Many of the laws, regulations, and policies being assessed will likely contain a description of their purpose, goals and objectives, ideally with reference to health and human rights. They may clarify the commitment to goals such as reducing maternal and perinatal mortality, improving maternal and perinatal health, ensuring gender equality, and upholding the sexual and reproductive health rights of women, thereby driving action.

MPDSR laws or regulations may aim to establish an effective monitoring and surveillance system to improve quality of care and reduce maternal and perinatal mortality, while promoting accountability and transparency by tracking the frequency of maternal and perinatal deaths. The stated objectives may include:

- documenting and addressing preventable causes of maternal and perinatal mortality;
- strengthening health systems;
- improving the quality of safe motherhood programming;
- identifying actions to prevent future shortcomings in access to high-quality essential maternal and neonatal health services; and
- enhancing maternal and perinatal death monitoring and evaluation.

Specific objectives may include creating a clear chain of enforceable institutional responsibilities for reporting and the establishment of mechanisms relating to confidentiality and medical liability while respecting and protecting access to justice for the community.

See Part 1 Section **3 Strengthening MPDSR-related laws and regulations** *for further information.*

2.2.1 Do existing laws, regulations, policies, and/or proposed or draft measures set a clear goal, purpose, and objectives for MPDSR?

2.2.2 Do existing laws, regulations, policies, and/or proposed or draft measures specify a health and human rights-based approach? In particular, do they refer to the health and human rights principles of accountability, participation, transparency, multidisciplinary cooperation, access to justice and remedies, confidentiality, privacy and autonomy in relation to establishing an MPDSR system?

2.3 MPDSR implementation

Effective implementation of MPDSR is essential for improving quality of care and reducing preventable maternal and perinatal mortality. The efficacy of MPDSR depends crucially on the timely provision of information about the occurrence of maternal and perinatal deaths and the prompt review of the causes and circumstances of those deaths. Establishing a legal or regulatory mandate for reporting these deaths is essential for taking action to prevent future occurrences.

Mandatory reporting promotes accountability and transparency by revealing the true scale of maternal and perinatal deaths, which may otherwise be underreported in death certificates. It involves establishing a clear chain of institutional responsibilities for reporting, and facilitates the "response" component of MPDSR. This enables timely interventions to address modifiable factors at healthcare facilities and within the community.

The following questions can be used to identify specific areas where implementation gaps may exist, allowing for targeted interventions to improve the effectiveness of MPDSR laws, regulations, and policies.

See Part 1 Section **5.1 Mandatory death notification** *for further information.*

2.3.1 Do the laws, regulations, and/or proposed or draft measures mandate the reporting of maternal and perinatal deaths?

2.4 Establishing an MPDSR system

A law or regulation may establish a legal duty to conduct maternal and perinatal death reviews, requiring the creation of institutional structures and procedural frameworks. This ensures a clearly defined accountability mechanism for a legally empowered institutional process for conducting maternal and perinatal death reviews. An MPDSR system may include the following elements, as outlined in WHO technical guidance:

Identification and notification:

- ongoing identification of suspected maternal and perinatal deaths in facilities (maternity and other wards) and communities; and
- immediate notification to the appropriate authorities within 24 hours for facilities and 48 hours for communities.

Review of maternal and perinatal deaths by local maternal and perinatal death review committees:

- examination of medical and non-medical contributing factors that led to the death;
- assessment of preventability and development of recommendations for preventing future deaths; and
- immediate implementation of pertinent recommendations.

Analysis and interpretation of aggregated findings from reviews:

- reviews are conducted at the district level and reported to the national level; and
- priority recommendations for national action are based on the aggregated data.

Respond and monitor response:

- implementation of recommendations made by the review committee and those based on aggregated data analyses;
- actions can address problems at the community, facility, or multisectoral level; and
- monitor and ensure that recommended actions are being adequately implemented.

Laws or regulations can mandate the creation of institutional bodies at national and subnational levels specifically responsible for conducting maternal and perinatal death reviews, including steering or coordinating committees and MPDSR committees. The composition of these committees is crucial from the perspectives of accountability, participation, and transparency.

Furthermore, laws and regulations can provide for institutional mechanisms for oversight, such as by parliament, not only to review the effective implementation of the system but also to mandate financial means for its functioning.

See Part 1 Sections **5 Elements of MPDSR laws and regulations**, **5.1 Mandatory death notification**, *and* **5.2 Establishing legally mandated MPDSR committees** *for further information.*

2.4.1 Do the laws, regulations, and/or proposed or draft measures make the reporting of maternal and perinatal deaths mandatory?

2.4.2 Do the measures create a clear chain of institutional responsibilities for the identification, notification, analysis, and monitoring of maternal deaths, as well as for the implementation of recommendations resulting from the MPDSR process?

2.4.3 Do the measures require the establishment of legally mandated maternal death review committees?

2.4.4 Are there specific provisions contained within the measures for national, subnational, and community-based committees?

2.4.5 Are there specific provisions contained within the measures on how national, subnational, facility, and community-based committees should communicate and exchange information?

2.4.6 Do the measures create a clear mandate for such committees?

2.4.7 Do the measures regulate the composition of such committees? Are there mechanisms to ensure a multidisciplinary approach to setting up MPDSR committees, guaranteeing the inclusion of different government sectors at both national and subnational levels, professional associations, human rights institutions, and affected communities?

2.4.8 Do the measures contain provisions for avoiding conflicts of interest when appointing members of maternal death review committees?

2.4.9 Do the measures specifically call for a transparent and participatory process, and, in particular, does it aim to ensure participation by civil society members, such as women's rights NGOs working on maternal mortality and representatives of affected communities?

2.5 Legal and regulatory solutions for a no-blame approach in MPDSR systems

MPDSR should be based on a no-blame approach. It is essential that laws and regulations protect the privacy and autonomy of individuals and families who participate as respondents in an MPDSR inquiry. At the same time, there should be remedial and redress mechanisms for the families of women and newborns, including stillbirths, who die from preventable causes, as well as disciplinary and liability mechanisms for healthcare workers. These may include, for example, court proceedings, patient rights mechanisms, and/or disciplinary actions. Legal guarantees should ensure the separation and coexistence of these systems, maintaining confidentiality in the review process while also ensuring access to justice for the families of the deceased and accountability for healthcare workers.

See Part 1 Section **5.3 Legal and regulatory solutions for a no-blame approach in MPDSR systems** *for further information.*

2.5.1 Do the laws or regulations ensure the confidentiality and privacy of MPDSR data and protect the privacy and autonomy of individuals and families who participate as respondents in an MPDSR inquiry?

2.5.2 Do the laws or regulations ensure remedial and redress mechanisms for the families of women and newborns who die from preventable causes, as well as disciplinary and liability mechanisms for healthcare workers? This may include, for example, court proceedings, patient rights mechanisms, and/or disciplinary actions?

2.5.3 Do the laws or regulations ensure the separation and coexistence of these systems, including maintaining confidentiality in the review process through legal safeguards for MPDSR participants, ensuring access to justice for the families of the deceased woman or the newborn, and providing disciplinary and liability mechanisms for healthcare workers?

2.6 Training

MPDSR laws, regulations, and policies should aim to secure the expertise needed for effectively collecting and assessing data within MPDSR systems. They can designate resources and mandate training to ensure a sufficient number of experts are available for the MPDSR system to function optimally. The quality of the MPDSR review process and the subsequent development of actions to address gaps depend significantly on the ability of reviewers to correctly assign causes of death. Adequate training enhances the reliability and effectiveness of MPDSR, a key quality improvement intervention.

See Part 1 Section **5.4 Training** *for further information.*

2.6.1 Do the laws, regulations, or policies establish mechanisms to ensure training concerning MPDSR processes and procedures?

2.7 Dissemination of MPDSR review results

Regular reporting and dissemination of MPDSR review results are essential for learning from the process and for accountability. Reporting and dissemination promote accountability and wider participation.

Reporting can be done through an annual report that includes data, findings, recommendations, and the implementation of the previous year's recommendations. This process can occur in various stages and formats. The law, regulation, or policy can mandate national and provincial stakeholder discussion meetings to identify key recommendations arising from the data prior to the publication of the reports. This way, the process begins as an internal inquiry and becomes a public issue for wider discussion. Clear responsibilities for public dissemination can be set by the law, regulation, or policy.

See Part 1 Section **5.5 Reporting and dissemination** *for further information.*

2.7.1 Do the measures require the compilation and dissemination of reports at regular intervals at national and subnational levels, making them available to various stakeholders, including healthcare professionals and affected communities?

2.7.2 Do the measures mandate national and provincial stakeholder discussion meetings to identify key recommendations arising from the data prior to the publication of the reports?

2.7.3 Do the measures set clear responsibilities for the public dissemination of the MPDSR reports?

2.8 Oversight

The establishment of institutional frameworks for conducting maternal and perinatal death reviews is an exercise of executive authority that must be democratically accountable. Legislatures play a crucial role in ensuring that laws, regulations, policies, and strategies positively impact maternal and perinatal health.

Accountability to legislatures can be ensured through various mechanisms, including regular ministerial reports, appearances before legislative committees, public hearings, commissions or inquiries, and field visits. In countries with devolved legislative powers, accountability mechanisms should also be devolved accordingly. Oversight mechanisms can support implementation by establishing legal means, including the allocation of financial resources for effective implementation.

See Part 1 Section **5.6 Oversight** *for further information.*

2.8.1 Do the measures and/or proposed or draft measures establish institutional mechanisms for oversight (e.g., by parliament) of MPDSR implementation?

2.8.2 Do the measures and/or proposed or draft measure require the allocation of appropriate financial resources to establish and implement the MPDSR process?

Part 3: Process guide

1. Introduction

The Process Guide provides an overview of the legal and regulatory processes for conducting legal, regulatory, and policy assessments related to MPDSR. It outlines steps for advancing these processes, including potential amendments to laws and regulations. The guide offers practical advice on how to use this manual for developing or revising MPDSR-related legislation or regulations. MPDSR is a quality improvement intervention that depends on an enabling environment to contribute to quality of care improvement efforts to reduce preventable deaths. This manual should be applied within a multistakeholder and participatory framework. All legal and regulatory development or amendment processes must adhere to principles of participation, accountability, and transparency, ensuring the involvement of relevant stakeholders.

The manual aims to support various state actors in strengthening legal and regulatory frameworks for MPDSR. This may involve applying existing laws to MPDSR, creating new regulations, or amending current ones to provide a legal and regulatory foundation for establishing and operating maternal and perinatal death surveillance and review systems. A well-structured process is essential for the effective development and implementation of MPDSR-related legislation or regulations. The following steps are recommended for this purpose.

2. MPDSR legal and regulatory strengthening process

Phase 1: Initiating a legal, regulatory, and policy assessment

- Step 1: Establishing leadership
- Step 2: Legal, regulatory, and policy assessment
- Step 3: Establishing commitment through a multistakeholder process
- Step 4: Early engagement with parliamentarians
- Step 5: Multistakeholder consultations

Phase 2: Applying, amending, or drafting and adopting measures

- Step 6: Official initiation of the legal or regulatory process
- Step 7: Drafting the laws and regulations
- Step 8: Public consultation
- Step 9: Adopting the laws and regulations

Phase 3: Dissemination of MPDSR frameworks

- Step 10: Public dissemination
- Step 11: Implementation

2.1 Phase 1: Initiating a legal, regulatory, and policy assessment

Step 1. Establishing leadership

Designation of a national focal point agency

Designating a focal point within the national government is essential for strengthening the legal and regulatory framework, including assessing existing laws, regulations, or policies and developing new laws or regulations or amending existing ones. Typically, a maternal or child health unit within the Ministry of Health (or equivalent) often serves as this focal point. This unit can play a crucial role in involving other ministries and key state actors in establishing the MPDSR system and engaging with civil society. The designated agency will be responsible for ensuring that the process thoroughly addresses national, subnational, facility, and community-based issues. Leadership in such an initiative may vary from country to country.

Establishing a steering committee

Following the designation of a national focal point, a government- or parliament-led steering committee may be established. Its role may include:

- ensuring that different governmental and nongovernmental actors are involved in the legal and regulatory strengthening process;

- facilitating dialogue with various stakeholders and consultations throughout the process;
- guiding the development of human rights-based approach in the laws or regulations; and
- ensuring the transparency of the legal or regulatory strengthening process.

A national focal point and the steering committee can also ensure effective communication and a productive working relationship with parliamentarians. Importantly, the steering committee can play a crucial role in communicating a comprehensive legal and regulatory framework around MPDSR, including any changes that may be made in a law or regulation or if a new law or regulation may be developed.

The steering committee may include representatives from:

- the relevant department of the Ministry of Health;
- other sectors of the government, such as the Ministry of Health or Ministry of Child Affairs;
- parliamentary committees related to women's and children's health issues;
- professional associations (e.g., national obstetrics, gynecology, and midwifery associations); and
- women's, newborn's, and children's rights NGOs, as well as maternal and perinatal health and rights organizations.

Where applicable, coherence between national and subnational coordination must be ensured.

Step 2. Legal, regulatory, and policy assessment

Assessing laws, regulations, and policies related to maternal and perinatal health, specifically MPDSR, is a crucial step in strengthening, developing, or amending MPDSR-related legal and regulatory frameworks. As explained in Part 1, **4.1 Assessment of existing laws, regulations, and policies**, MPDSR laws and regulations cannot be developed or amended in isolation from other laws, regulations, and policies related to maternal and perinatal mortality reduction in the health sector, other sectors, or the broader legal system. Therefore, it is necessary to consider which instruments are relevant for MPDSR. Relevant instruments might include the constitution, civil code, gender equality law, sexual and reproductive health law, administrative law, common law, customary law, and court decisions.

It is possible that the assessment may conclude that there is already an existing strong legal and regulatory framework that protects maternal and newborn health; hence, the development of new measures may not be necessary, but existing legislation and regulations should be applied to MPDSR policies. However, existing legal and regulatory frameworks may not sufficiently cover all aspects of MPDSR, so the amendment of existing laws or regulations, or the development of new laws or regulations may be necessary.

This assessment helps identify any inconsistencies between existing laws, regulations, and policies that the amendment or development of a new law or regulation can address. It also provides an opportunity to avoid overregulation and harmonize existing laws and regulations with new MPDSR laws or regulations.

For instance, since MPDSR-related laws and regulations should facilitate a no-blame approach, any MPDSR development must consider the broader legal contexts of privacy, confidentiality, and data protection, legal and criminal liability, and laws guaranteeing access to justice, remedies, and redress.

The legal and regulatory assessment can include drafting a suggested amendment or a new law or regulation for consideration in the multistakeholder process. The Checklist in this manual can assist in that process.

Step 3. Establishing commitment through a multistakeholder process

To achieve a transparent, accountable, and participatory legal and regulatory process, it is essential to build consensus on the need to develop and/or revise MPDSR-related laws or regulations in order to achieve a transparent, accountable, and participatory legal and regulatory process. A comprehensive approach requires the engagement of multiple state- and non-state actors with complementary mandates and responsibilities. These actors must be free from conflicts of interest and be led by a designated state body to secure sustainable political commitment and responsibilities. Participation by women's, newborn's, and children's agencies, as well as affected communities, is crucial for the legitimacy and successful implementation of the legal and regulatory process.

The Ministry of Health plays a vital role in this process, ensuring that laws or regulations are part of the ongoing effort to improve quality of care and reduce maternal and perinatal mortality in the country. Other ministries, such as the Ministry of Justice and Ministry of Gender and Women's and Children's Health, along with nongovernmental actors, such as human rights, women's or children's rights commissions, can help ensure that the law or regulation adopts a human rights-based approach. Consulting with the Ministry of Finance is necessary to ensure adequate resources are allocated for implementing the legally mandated MPDSR system.

Involving health system actors, such as obstetrics and gynaecology and midwife associations, is crucial for identifying health system and human resource barriers and ensuring an effective implementation vision. Including civil society and community representatives legitimizes the process, as their voices are essential for effective implementation of the MPDSR system and the elimination of maternal and perinatal mortality.

Step 4. Early engagement with parliamentarians

Early engagement with parliamentarians is essential when planning for an amendment or the development of new laws or regulations. They play a crucial role in amending, adopting, and implementing laws or regulations by holding national governments accountable through their oversight functions. Parliamentarians are critical in monitoring laws, policies, and strategies to ensure their effective implementation.

Budgetary appropriation is a significant method for ensuring that funds are allocated where they are needed most and in the most effective manner. Parliamentarians are key to ensuring adequate funding for maternal, newborn, and child health, supporting cost-effective interventions. By addressing maternal, newborn, and child health, parliamentarians can demonstrate that these issues are central to a nation's development and essential to the well-being of their constituents. Their action should reflect the situation in their constituencies and they should convey their constituents' voices to the national level. Through their core functions, parliamentarians can strengthen MPDSR-related actions and interventions, highlighting the importance of maternal, newborn, and child health in national development agendas.

Step 5. Multistakeholder consultations

A multistakeholder consultation may be convened to introduce all stakeholders to the objectives and purpose of the proposed MPDSR law or regulation and to discuss its elements and implications. These consultations can take place not only at the national level but also at subnational levels to incorporate decentralized or federal perspectives on the measure's design and implementation realities. Involvement of subnational actors in national consultations is essential to ensure their perspectives are represented. Such consultations foster the legitimacy, participation, and transparency of the legal or regulatory process. They support the application or amendment of existing measures or the development of new laws or regulations and provide an opportunity to view it within the broader

context of the legal, policy, programmatic, and social environment of existing maternal and perinatal death reduction efforts.

State actors, such as ministries of health, justice, gender and women's and children's health, and nongovernmental actors, along with women's and children's commissions, civil society and community representatives, as well as parliamentary bodies and parliamentary groups on women's and children's health and human rights, and other relevant stakeholders can collaboratively assess whether current laws, regulations, or policies adequately mandate and support effective implementation of MPDSR systems.

Together, they can identify any legal, regulatory, and policy gaps that hinder effective implementation and determine whether and how MPDSR-related laws or regulations could help improve quality of care and reduce preventable maternal and perinatal mortality and morbidity. They can also commit to ensuring that appropriate laws or regulations are introduced and mandate the formation of a legal or regulatory committee to assist and oversee the preparation of amendments or a new law or regulation.

The law, regulation, and policy assessment (based on the use of this manual) should be distributed to all stakeholders in advance of their meetings. Presentations should be prepared to highlight both the public health and human rights objectives of developing MPDSR legislation or regulations. Sufficient time should be allowed for meaningful discussion and consensus-building on the next steps.

The multistakeholder process may result in one the following decisions:

- strengthening and applying existing legal and regulatory frameworks related to maternal and newborn health, specifically applying these to MPDSR;
- deciding that developing a new law is unnecessary, but opting to amend existing laws or regulations or create new ones (this is typically the role of the executive branch, usually the Ministry of Health or Ministry of Justice);
- amending existing laws or regulations to make them more specific to MPDSR systems; or
- developing a new law or regulation that provides more specific guidance on MPDSR.

2.2 Phase 2: Applying, amending, or drafting and adopting measures

Step 6: Official initiation of the legal or regulatory process

Amending an existing law or regulation or developing a new MPDSR law or regulation should incorporate a transparent and consultative process. The process may be initiated by various actors and led by entities such as the steering committee discussed above. Parts One and Two of the manual can aid in developing a new law or regulation or amending an existing one. If the MPDSR legal, regulatory, and policy assessment has already led to or revealed a draft law, regulation, or amendment, that draft should be taken into consideration.

Step 7: Drafting the laws and regulations

A drafting team can be appointed by the body coordinating the development or amendment process to ensure effective and expertise-based development of the law, regulation, or amendment. This team should bring together expertise in maternal and perinatal health and human rights, knowledge of existing laws, regulations, or policies related to maternal and perinatal health, and an understanding of health systems concerning maternal and perinatal death reduction and broader sexual and reproductive health and rights.

The drafting team must ensure that the proposed measure or amendment aligns with the broader framework and strategy for maternal and perinatal mortality reduction. A legal or regulatory assessment report should be carried out to review existing laws, regulations, policies, human rights, public health commitments, and information on implementation, good practices, and barriers. These considerations are essential for aligning the new law, regulation, or amendment within the wider domestic framework.

Parts One and Two of the manual can aid in developing the law, regulation, or amendment. The drafting team must consider any necessary adaptations to ensure that the draft law, regulation or amendment reflects national specificities. For instance, they should consider how the law or regulation responds to community needs and systematically addresses the needs and rights of marginalized populations. If the MPDSR legal, regulatory, and policy assessment has already revealed a draft law or amendment, the legal or regulatory team may use that as a basis for their drafting work.

The finalized draft should be presented to the relevant domestic entity and follow the specific procedures for reviewing draft legislation or regulations, which may vary across countries.

Step 8. Public consultation

Presenting the draft law, regulation, or amendment to the multistakeholder group provides an important opportunity for receiving valuable feedback, ensuring transparency, and legitimizing the legal and regulatory process. Before a committee debates and votes on decisions regarding a proposed law or regulation, it typically holds a public hearing in collaboration with the coordinating government body. A public hearing may be one of the most important public milestones in the legal and regulatory process. These hearings also provide ideal opportunities to attract media attention to the proposed law or regulation and the underlying issue.

Step 9. Adopting a law, regulation, or amendment

Following consideration of the proposed law, regulation, or amendment, public hearings are usually proposed. The Parliament and the government, depending on whether the change is in a law or a regulation, can ensure that the new law or regulation is aligned with the broader constitutional, legal, and policy framework for improving maternal, newborn, and child health. The extent of parliament's mandate in law or regulation development varies by country. However, many constitutions grant parliament the authority to scrutinize, amend, or reject a draft law or regulation that is inconsistent with international public health standards and human rights obligations, standards, and guidance. On this basis, parliamentarians can enact laws, regulations, and policies that guarantee equitable access to health for all women, newborns, and children, including those who are vulnerable or marginalized. They play an important role in ensuring that the measure promotes health rights for all women, newborns, and children, including sexual and reproductive rights, and promotes equity in healthcare provision.

2.3 Phase 3: Dissemination of MPDSR frameworks

Step 10. Public dissemination

The dissemination of new law or regulation is a crucial aspect of the legal and regulatory process. This can be achieved through collaboration with parliamentary and government bodies responsible for dissemination, the Ministry of Health, and other ministerial bodies, and with civil society actors for women's, newborn's, and children's rights, sexual and reproductive health rights, and maternal and perinatal rights, at both national and subnational levels. Dissemination efforts not only inform about the content of the law or regulation but also include consultations and training for stakeholders responsible for its implementation. The steering committee, in partnership with relevant parliamentary and governmental bodies, plays a key coordinating role in these dissemination efforts.

Additionally, dissemination activities need to be planned at national, subnational, institutional, and community levels.

Step 11. Implementation

Various stakeholders play critical roles in the oversight and implementation of new laws or regulations. Parliamentarians, for instance, are not only responsible for adopting laws or regulations but also for overseeing its implementation. They can hold national governments accountable through their legislative and oversight functions. Parliamentarians have a crucial role in monitoring the effective implementation of the MPDSR measure, including ensuring that appropriate funds are allocated for its implementation in the most effective way. Oversight of effective implementation can involve receiving regular reports on activities from the executive arm of government, tabling parliamentary questions to members of the executive arm, overseeing implementation through constituency visits, and establishing and participating in special committees and commissions of enquiry. Working with local administrative, religious, and community leaders, parliamentarians play an essential role in demonstrating commitment to maternal and perinatal health.

Parliamentarians also represent the views and concerns of their constituents in parliament and provide constituents with information on national and parliamentary issues. This role requires parliamentarians to receive feedback from their constituents regarding the effectiveness of MPDSR laws or regulations. By responding to the actual situation on the ground and ensuring that the voices of their constituents—including nurses, midwives, community healthcare workers, and community representatives, are heard at the national level, parliamentarians can show that maternal, newborn, and child health is at the core of a nation's development and central to the well-being of its people. *(141)*

3. Limitations to adopting MPDSR laws and regulations

The adoption and implementation of laws and regulations face several limitations, including resource constraints, enforcement challenges, lack of political will, and inadequate community engagement. Key barriers and potential strategies to address them include:

1. **Resource constraints**: Limited financial and human resources can hinder the effective implementation of MPDSR laws and regulations. To mitigate this, countries can explore avenues such as international funding, public-private partnerships, and more efficient resource allocation within existing health budgets.
2. **Enforcement challenges**: The lack of robust mechanisms to enforce MPDSR laws and regulations can undermine their effectiveness. Strengthening enforcement requires building the capacity of health professionals, enhancing monitoring systems, and establishing accountability frameworks to ensure compliance.
3. **Lack of political will**: Political and governance issues, such as competing priorities and inadequate leadership commitment, can stall the adoption of MPDSR laws and regulations. Overcoming these barriers involves fostering multi-stakeholder collaboration, advocating at high levels of government, and integrating MPDSR into broader health policy agendas.
4. **Insufficient community engagement**: A lack of community involvement in the legal and regulatory process can result in ineffective measures, poor data quality, and inadequate responses. To enhance community engagement, strategies such as information campaigns, workshops, collaborative platforms, and incentives for participation can be employed, ensuring that communities actively participate in the MPDSR process.
5. **Alignment with existing systems**: The development of new MPDSR laws or regulations should align with the current implementation of MPDSR systems to avoid duplication of efforts and ensure a cohesive approach. Continuous evaluation and integration of feedback from existing MPDSR practices will support the effective enactment of new laws and regulations.

By addressing these limitations, stakeholders can create a more supportive environment for implementing MPDSR laws and regulations.

Conclusion

This manual can be a vital resource for countries committed to improving quality of care and enhancing maternal and newborn health outcomes, providing a comprehensive framework that guides decision-makers in enacting and implementing effective MPDSR laws, regulations, and policies. These frameworks establish a structured approach to monitoring and responding to maternal and perinatal deaths, which remain significant public health issues globally. Robust MPDSR frameworks are essential for systematic surveillance, reporting, and response, empowering healthcare systems and communities to collect accurate data, analyze causes, and implement interventions. This ensures accountability among all stakeholders and supports the fulfillment of international, regional, and national human rights obligations.

MPDSR laws and regulations contribute fulfilling to numerous health and human rights objectives. Figure 2 below summarizes the potential impacts derived from implementing MPDSR laws or regulations. At the centre of the diagram is the core element: MPDSR laws and regulations, which provide the foundational basis needed to mandate the collection, reporting, and analysis of maternal and perinatal deaths. These measures establish requirements for healthcare providers, facilities, and authorities to participate in MPDSR processes.

MPDSR laws and regulations serve as the cornerstone for achieving significant improvements in maternal and newborn health outcomes through systematic review, accountability, policy development, quality improvement, stakeholder engagement, mortality reduction, data protection, health system strengthening, and access to justice.

Figure 2: Role of MPDSR Laws or Regulations

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